

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

WHITNEY HORNICK, PAMELA
HORNICK, DONALD HORNICK by his
litigation guardian WHITNEY HORNICK,
and DYLAN HORNICK by his litigation
guardian WHITNEY HORNICK

Plaintiffs

- and -

WARREN KOCHINSKY and U.S.A.
TRUCK INC.

Defendants

)
)
) James E. Allin, for the Plaintiffs
) Whitney Hornick, Donald Hornick
) and Dylan Hornick
) Stephen J. Fuerth for Pamela Hornick
)
)
)

) Robert E. Barnes, Q.C., Harvey T.
) Strosberg, Q.C., John C. Holland, and
) Jay S. Strosberg, for the Defendants
)
)
)

) **HEARD:** February 26, 27, 28, 2003;
March 3, 4, 5, 6, 7, 2003; April 11,
2003; June 4, 2003; September 8, 9,
10, 11, 12, 16, 17, 2003 and April 19,
20, 22 and 23, 2004.

Thomson J.

JUDGMENT

[1] This action arose as a result of a motor vehicle accident that occurred in the westbound driving lane or shoulder area of the 401 Highway at or near Chatham, Ontario. A fully loaded tractor-trailer traveling westbound at approximately 100 kph in the driving lane or on the shoulder or both crashed into the rear of the stopped or slow moving maintenance dump truck being driven by the plaintiff Whitney Hornick [Hornick] in the driving lane or on the shoulder or both. He was legally working on routine maintenance of

the highway and was in the middle of a convoy of maintenance vehicles heading west on the 401.

[2] Hornick claimed he suffered physical, psychological or other mental injuries as a result of the accident. The other plaintiffs were family law claimants.

AGREED FACTS:

[3] An agreed statement of facts was filed concerning the insurance details about the defendant's truck. It is not necessary to specifically refer to them.

[4] The defendants admitted they were liable to the plaintiff for whatever damages the plaintiff could prove.

[5] The parties agreed the threshold test for damages was as set out in section 267.5(5) of the *Insurance Act* as follows, the plaintiff must suffer "a permanent serious impairment of an important physical, mental or psychological function" in order to be successful in his claim for damages.

[6] The physical injuries suffered by the plaintiff, given the plaintiff's occupation, crossed the threshold and the injuries resulted in a permanent serious impairment of an important physical function.

[7] The defendants were protected parties by virtue of section 267.5(1) and therefore were entitled to deductions of \$15,000 against Hornick's general damages assessment and \$7500 against each *Family Law Act* [FLA] award.

[8] There was no issue with the settlement with State Farm Insurance.

ISSUES and ANSWERS:

A. Was Hornick a credible witness? Yes.

B. Regarding physical function and contributory causation.

- [9] 1. Was there a permanent serious impairment of an important physical function?
Yes.
2. What was the degree of that serious impairment of an important physical function? Significantly serious.
3. Was there any contributory causation to the permanent serious impairment of the important physical function? No.

C. Regarding mental or psychological function.

1. Was there a permanent serious impairment of an important mental or psychological function? Yes.
2. What was the degree of that serious impairment of an important mental or psychological function? Moderate
3. Is there a difference between impairment of a mental or psychological function? Yes in law but perhaps not to the psychologists or psychiatrists.
4. Regarding the credibility of Dr. Sweeney and Dr. Shapiro whose evidence was more credible? Dr Sweeney's.
5. What import, if any, was the SPECT scan in this determination? Very helpful

D. Regarding mitigation and Hornick's motivation to go back to work.

1. Was there a failure to mitigate? No
2. Was Hornick motivated to go back to work? Yes
3. Was Hornick malingering? No.

E. Regarding the Family Law Act [F.L.A.] claims.

1. Does Pamela Hornick have a F.L.A. claim and if she does what is the quantum of the damages after deductions? \$37,500.00
2. Do Donald and Dylan have F.L.A. claims and if so what is the quantum of their damages after deductions? \$37,500.00 each.

F. Regarding Hornick's damages.

1. When does a civil trial start? In a jury trial, when the case is first put in the hands of the jury, for instance, where opening instructions are given to the jury by the judge or counsel make opening addresses or, in a non jury trial, when counsel gives the opening address or if no address, the calling of evidence.
 1. What are Whitney Hornick's general damages, if any? \$125,000.00
 2. What was Whitney Hornick's past-lost income, if any? This amount will be calculated in accordance with the formula in the past lost income portion of the judgment.

3. What will be Whitney Hornick's lost future income? This amount will be calculated in accordance with the formula in the lost future income portion of the judgment.
4. What will be Whitney Hornick's lost handyman capacity to earn income and what would that income be? This claim was abandoned by the Plaintiff.

G. Regarding credits, long-term disability benefits and future statutory accident benefits payments.

1. What are the credits, if any, to be deducted from Whitney Hornick's claims? As set out in exhibit 59, \$157,318.00
2. What happens to Whitney Hornick's long-term disability benefits? The defence will have the right to request an assignment of these benefits if they are payable to Hornick.
3. What happens to Whitney Hornick's future Statutory Accident Benefits payments? This matter was not fully discussed during the trial and I may be spoken to when the matters of pre judgment interest, costs and GST are dealt with later.

OVERVIEW:

[10] Whitney Hornick and Pamela Hornick [Pamela] were married on October 6, 1984 and were the parents of Donald Whitney Hornick [Donald] born October 9, 1989 and Dylan Richard Hornick [Dylan], born February 9, 1984.

[11] Warren Kochinsky [Kochinsky] resides in the State of Texas, in the United States of America. On December 17, 1997 he was operating a fully loaded tractor-trailer truck on highway 401 at or about 100 km. per hour in the curb or shoulder lane close to Chatham, Ontario. The truck was owned by U.S.A. Truck Inc.

[12] On the same date and at the same time Hornick was operating a single axle crash barrier dump truck with a large sign arrow slowly on the 401 highway in the driving lane or shoulder area or both doing required road repairs.

[13] Kochinsky ran into the rear end of Hornick's truck causing an extremely violent crash. Damage to the vehicles was approximately \$150,000.00.

[14] Pamela, Donald and Dylan claim damages pursuant to the provisions of the *Family Law Act*.

FACTS:

[15] Because there was an allegation that Hornick may have been trying to trick or mislead the experts or over exaggerate his physical or mental problems or feign his psychological responses, it was necessary to fully review all the civilian and expert evidence on both sides to arrive at the appropriate conclusion on this allegation. It became a battle of the experts.

[16] On the date of the accident, **Paul Atkinson** was driving a truck in the East bound driving lane of 401 Highway east of Highway 40, east of Chatham, Ontario. His truck contained cold patch material to fix holes or cracks in the pavement and was first in a line of three trucks doing this work. The plaintiff was operating a crash barrier barrel truck behind the other three trucks. It had an arrow board on the back that directed traffic from the driving lane into the passing lane. The plaintiff's truck had a large number of 50 gallon barrels attached at the rear of the truck which were designed to act as a crash barrier to protect both him and the truck in front of him from being hurt in a crash. A pickup truck was traveling approximately 1000 feet behind the Plaintiff's truck. It had a large board on the back with an instructing arrow sign indicating that traffic should move from the driving lane into the passing lane.

[17] As he was driving along he heard a radio communication from the driver of the pickup truck 1200 feet behind him that indicated to the plaintiff that the tractor-trailer was "not getting over". He looked in his side view mirror and saw the Kochinsky tractor-trailer hit the plaintiff's truck. Paul described it as a "terrific crash" and that it "looked like an explosion".

[18] The two trucks came to a stop on the shoulder of the Highway. He called for the police, fire and ambulances to attend. He was just going to get out of his truck when he looked and saw the plaintiff coming towards him, walking on his own with a man on each arm assisting him. The plaintiff got in the truck and seemed lucid but bewildered. It took him some time to convince the plaintiff to go in the ambulance as he kept saying, "I don't want to lay in there". Paul insisted that the plaintiff go in the ambulance and eventually, after learning that the plaintiff could sit in the ambulance, the plaintiff did in fact go in the ambulance.

[19] The pictures shown at tabs 5 and 6 of exhibit 1 fairly represent what the plaintiff's truck looked like originally and after the crash. They also indicate the damage to the defendant's truck.

General background concerning Hornick both before and after the accident:

Before:

[20] The plaintiff's two brothers, mother, sister-in-law and coworkers and friends testified at the commencement of the trial about the plaintiff from the time he was a little boy until the present. Their evidence was consistent throughout about his physical and emotional state during those times.

[21] The family was close and in contact with each other either by phone or in person on a regular basis both before and after the accident. The only family members who were not in regular contact with the plaintiff were his brother Duane and sister-in-law Judy.

[22] Physically, the plaintiff was a normal male who grew up being able to do all of the things a male would be expected to be able to do. He could run, play, lift, bend, cut grass, do farm chores, lift both light and heavy objects, drive farm machinery and do it all without any physical limitation in any way. He did not limp and was able to do complicated physical work.

[23] Mentally and emotionally, he was good in school, finished grade 11, was able to be a partner in a service garage business with his brother, was able to think clearly, reason things through and remember relatively complicated tasks. He was polite, never swore, was emotionally stable, got married at age 24 and led a relatively normal life.

[24] **Paul Atkinson** worked with the plaintiff both before and on the date of the accident. Before the accident, the plaintiff would work with him doing highway maintenance work that consisted of cutting grass, cutting weeds, fixing signs, plowing snow, patching highways and other related work. When they cut grass and weeds they would often have to walk on uneven ground with gravel, stones, rocks and other materials that would cause footing to be sometimes perilous. Hornick had no difficulty doing those chores.

[25] One particular example that Paul spoke about was how they had to erect signs on the sides of the highways that had been damaged by cars or vandals. The signs were supported by 6" by 6" by 12' or longer posts. Two men would be required to carry these very heavy posts. Paul and the plaintiff did this kind of work. It would require them to dig around the old posts, pry them out of the hole, cart to them away and load them into their truck. They would then have to use a manual posthole digger, dig a new hole, place the new post, level it and tamp it into place. They would then have to attach the 4 by 8 sign to the posts. This was very heavy work. At no time while they worked together before the crash did the plaintiff complain about aches or pains in his neck, back or legs or complain about headaches. He was able to do his work without protest and never "sherked" his job responsibilities. The plaintiff never limped during the year or so that he worked with Paul.

After the accident:

[26] After the crash, Hornick complained of physical pain in his legs and back. He suffered serious headaches and complained about the front of his forehead being swollen. The headaches were regular and lasted for as long as four days.

[27] All family members indicated that the plaintiff limped after the accident and each attributed the limp to the crash. The plaintiff could not work, could not operate a lawn tractor for any period of time and was unable to follow instructions when told how and

where to cut grass at his relatives' homes. He could not run and chase his children, go on rides with them or go fishing and snowmobiling with his children and his brother for long periods of time as they all did before the accident.

[28] Emotionally, he became snappy and short with his family members in situations where he became frustrated. This happened often and as a result of simple things that occurred in his life.

[29] His brother **Murray** allowed the plaintiff to live with him for approximately 1 1/2 years after the accident. He noticed the decline in the plaintiff's physical, emotional and mental state on a daily basis. The plaintiff could not remember relatively simple things like where he left his calendar of appointments and future events. He would forget date and times of appointments and events. One example that stuck out in his mind was that Hornick would let the dogs outside for a while in the wintertime but he would forget the dogs. Murray would come along and the dogs were outside shaking and freezing because Hornick had forgotten he had let the dogs out.

[30] At times, he would become so frustrated that he would swear at his mother, something that he never did before the crash. Murray was somewhat emotional in the witness stand when he talked about the increase in Hornick's anger and the change in his brother. He was not the same brother.

[31] **Sadie Hornick** was Hornick's mother. Her evidence was clear that Hornick was a worker and had always been a worker. He had good jobs, went to work regularly, worked at Duo-Matic for 18 years as well as working on the family farm helping his father.

[32] He lived with her after the accident and she said he was continually "fustrated" [sic frustrated]. It was clear from her evidence that there had been a very significant change from the way he was as a son and worker before the crash to this "fustrated" individual who was continually around her home afterward.

[33] She was a decent, honest, truthful woman who was concerned about her son and the change that occurred in him from December 17, 1997.

[34] Hornick's brother **Dwayne and Judy Bowman** said much the same thing but did not have the benefit of actually living with him after the crash like Sadie and Murray did. They, too, felt there was something wrong. Hornick the worker had changed and had become an angry, frustrated and forgetful man and they too concluded there was something wrong with him after the crash.

[35] **Chris Ouellette** was a co-worker who worked at Duo-Matic with Hornick for a number of years. He was very straightforward and confirmed Hornick was a worker. Hornick supervised 35-40 employees on three different lines who were assembling furnaces with many different parts at many workstations. It was his responsibility to make those lines "hum". He had to have all of the parts and all of the people working together in

concert at the different workstations. Ouellette was a candid, forthright, decent, honest witness who told the court that there was something wrong with Hornick after the crash.

[36] I am satisfied that all of this evidence has a ring of truth about it and it is easy to conclude on balance that Hornick was a hard working, decent, honest family man before the accident but afterward he was a changed man physically and mentally. It was also clear that he had some interpersonal problems with his wife that will be discussed later.

PARTICULAR EVIDENCE FROM THE PLAINTIFFS

Pamela Hornick [Pam]:

Pre-accident:

[37] Thirty-eight year old Pam testified that she completed her high school education at age 17 while in grade 12. She married Hornick and they spent their married life in Tilbury, Ontario living at 10 Park Lane where she still lived.

[38] Following the marriage, Pam worked part time outside of the home and, since 1989, had been working as a health care aid on a part-time basis. They planned to have a family and did.

[39] She described her life with her family as a normal marriage. She did the housework, cooked, maintained the home and looked after raising the kids as well as banking, paying the bills and working part time.

[40] Hornick helped with the cooking, cleaning, laundry, yard work, snow removal, grass cutting, outside cleaning of the windows and doing repairs around the house.

[41] The family [including the children] would play, swim, have campfires, ski-doo, skate, toboggan, bike, camp and ice fish together. They attended car shows in the summertime. They spent a lot of time socializing mostly with his family and frequently took holidays camping in Wheatley, Ontario. They organized celebrations for the children and took care of their children's physical and other needs.

[42] As a couple they were jointly engaged in the management of their financial affairs and made joint decisions about purchases. For example Hornick wanted to purchase a classic car and Pam co-operated by assisting in obtaining a loan for that purpose.

[43] Hornick was a strong man, completed heavy chores and was very capable of lifting big and heavy objects. She was aware that he had hurt his right knee playing hockey as a boy before they got together in 1983.

[44] He never took any time off work because of his right knee during the five years before the accident. He never complained of pain in his right knee but she could see him

rubbing it on the odd occasion and he might take Tylenol. He never walked with a limp at any time in the five years before the crash.

[45] When he worked in Wallaceburg at Duo-Matic he would have some bad days but when he began working at Miller's just before the crash, he enjoyed that job and was happy.

[46] Pam also reflected the reality of their relationship by testifying that her marriage was a normal one before the crash. However, she also agreed she had consulted a lawyer before the accident because of occasional matrimonial difficulties. When pressed in cross examination, she agreed that for about 10 years before the accident, Hornick would swear at her, call her names, argue heatedly with her, put her down, belittle her, make fun of her and showed little or no empathy towards her whatsoever. For instance, if she wanted to read in the evening, he would not understand that desire. He was jealous that she wasn't communicating with him. She agreed he was quick to anger before the accident. He was also angry with her family and repeatedly complained about her mother. He would get upset when his children would spend time with her family. He would erase telephone calls that have been left on the phone for her by her family. Notwithstanding these difficulties she decided to stick it out.

Post accident:

[47] After the accident, she first saw Hornick at about 7 p.m. in the hospital and sometime later they left together. Once at home he took drugs for pain and mainly sat around or laid on the couch. He would complain about his neck, back, knees, shoulder and awful headaches. She could tell by his facial gestures that he was in pain.

[48] Duties changed around the house and she would do the snow removal and cut the grass. He could load the washing machine and do the laundry. He no longer could look after the pool. Sometimes he would assist in cutting the lawns.

[49] She noted changes in his behaviour. He did not like dealing with doctors and filling in forms. It frustrated him. He was depressed and sad and slept more than usual. He was bitter because of the accident because it had disrupted their lives. She continued to work a bit.

[50] Hornick began to do some bizarre things. For instance twice he undid the battery in her car so she could not get it started. She confronted him and he said that it was his car and he could do what he wanted because it was in his name.

[51] His anger had a lot to do with their separation. He would get mad and stay mad. The kids would set him off especially if they came in the house with dirty shoes. Before the accident that would not have bothered him. Little things would bother him and he would curse and swear but he would not use profanity in front of the kids.

[52] Before their separation in 1999, Pam went away on a vacation with three of her girlfriends from high school. She had told Hornick about the trip a few months before she went and he was not happy. He told her that he hoped the plane would blow up with all of them in it. They were gone eight days and when she got back Hornick was not too receptive towards her. She was sitting in a chair downstairs when he came up behind her, tipped the chair over and she went flying. The next day he took a cigarette and burned her arm. He hid her souvenirs that she had brought back and broke some of them. The kids were not present during these altercations. He accused her of having an affair. One day she took a dust mop and hit him. On October 27, 1999 they separated and there has been and will not be any reconciliation.

[53] After separation she lived with her parents for a while. Then she went back to the matrimonial home with the kids and stayed there. At the date of trial Donald was living with his father, and Dylan was living with his mother. Donald left because of an argument with Pamela and has not lived with her since. Dylan has some health problems.

[54] Donald does not speak to Pamela, doesn't confide in her but does come over to her house for short visits. There is no discussion between Hornick and Pamela regarding parties, holidays or gifts and there is no discussion about school activities. Occasionally they do write notes back and forth about when the kids will visit.

[55] Hornick sold his one half interest in the matrimonial home to Pam's boyfriend who is now cohabiting with Pam and has done so since November 2002. They have a permanent relationship.

Whitney Hornick:

[56] The plaintiff was born Oct. 22nd, 1958, married when he was 24 years old and separated in fall of 1999. Donald and Dylan were his natural children.

[57] He has worked since he was a teenager at various jobs all of which required varying degrees of physical labour. He confirmed his work with Duo-Matic and Miller and that was confirmed by fellow employees from each place.

[58] On Dec. 17, he saw the tractor-trailer coming and said that it hit him very hard. He was injured and thought that he had been knocked out. He had a very sore head on the right side of his fore head over his right eye and the back of his head. He could not tell how long he was unconscious. He did not know where he was until he regained consciousness as he was sitting on the side of a vehicle. He blacked out again. He did not know how long.

[59] He then he recalled waking up in the hospital but also recalled ambulance personnel asking his name, the time and the date on the way to the hospital.

[60] He listed his pains and injuries from the top of his head to his feet as follows, namely; the back of his head, the front of his head, headaches, neck pain, upper back pain, left shoulder pain, lower back pain, knee pain and heel pain. He was dizzy and had trouble remembering and concentrating.

[61] The next day he called his family doctor, Dr. Patel. He was sent for an x-ray and was placed on medication. There were no broken bones. He later attended physiotherapy. It did help for a while. Over the years he received a number of CT scans and saw a variety of different doctor's and professionals. He regularly saw Dr. Patel and attended the gym regularly until 2002.

[62] He recalled some of the doctors and what his understanding was of their analysis of his problems. He was unable to give very many specifics. He was consistent in explaining the aches, pains and headaches as well as his problems with remembering, concentrating, frustration, forgetting and depression. Things were not going to way they used to. He said he had a short fuse and his nerves were not like they used to be. He testified that before the crash things were good but after the crash they were no good.

[63] In March of 1999, Dr. Archibald saw him about his headaches and memory loss. She told him that he had closed head brain injury. He saw her a couple of times.

[64] He became separated from his wife in October of 1999. It was his opinion that the separation had something to do with the crash. He had been receiving SABS but had been cut off. There was a shortage of money. When his wife's income tax refund arrived, she used it to go on holiday. Later, while looking around at his residence, he came across pictures of his wife with another man. He became very upset and some physical confrontation occurred later. He was charged, removed from the house without his personal belongings, put in jail but was later released on a peace bond to stay away from his house and his wife. He testified that his wife assaulted him. It was his view that his wife was not thinking about him or his two boys. They then separated.

[65] His work has already been described by others who worked with him and I don't intend to spend any more time describing any aspects of his work up to the crash except to add that he used to start the bus up at four thirty in the morning, pick his workers up at quarter to six and wouldn't get home till he had done his supervisor job, bused his fellow employees home, returned home at four thirty and finally parked the bus.

[66] Regarding his efforts to find a job, he indicated that the last time he visited the Employment Canada office in Tilbury was the week before he testified? He had also been there 2 weeks before that visit. He explained why he did not go every day as follows, "sometimes I don't feel like it. Sometimes, I don't feel good. Like today, I don't feel good" and the other problem was "parking".

[67] He was asked whether it had occurred to him that he might be able to do some other job, less rewarding than a truck driver. He asked why he had not been retrained and

then said he was “not going to look for anything less than that then. Why should I.” He then added again “Some days I don’t feel good. Some days I don’t feel like doing nothing” because he was hurting and could not do any kind of work at all.

THE EXPERTS:

For the Plaintiff:

Dr. Teasell:

[68] He was a duly qualified medical practitioner licensed to practice medicine in the Province of Ontario with a specialty in physical medicine. He was the Chair-Chief, Department of Physical Medicine and Rehabilitation at St. Joseph’s Hospital in London Ontario. As well, he was the city wide chief for all of rehabilitation and the chair of the University of Western Ontario Medical School Department of physical medicine.

[69] At the University the department was charged primarily with education and research as well as providing clinical care for patients. There was a core group of eight physicians and one PhD. As well there were part-time doctors and PhD’s as well as 12 additional research staff.

[70] Dr. Teasell was accorded status to give opinions concerning physical medicine with particular expertise in whiplash, back pain and chronic pain.

The dynamics of a motor vehicle accident:

[71] The doctor explained in some detail the dynamics of a rear end crash. More neck injuries occur with rear end collisions. Headrests were put in cars to try and prevent whiplash and they had reduced whiplash by about 25 percent. Most people don’t use their headrest properly. Studies have shown they don’t adjust them up properly and that when one sits in a car there is usually a significant difference between the back of the head and the head rest and as a result, it is not really effective. So, when someone is hit from behind, their neck snaps back very quickly and the head rest usually does not stop that. The head then comes forward and then back when it hits the head rest. If Hornick’s head had actually gone through the window, and it is not clear from the other data that it in fact did, that would have indicated a very severe neck hyperextension. However, the fact that it may not have gone through the back window does not preclude him from still having chronic symptoms or a chronic neck injury because of the initial neck hyperextension.

[72] The difference in speed between the vehicle struck [slow or stopped] and the striking vehicle [fully loaded tractor trailer at 100 kph] is called the “Delta V” that is, the acceleration that is generated to the struck vehicle. There is a relationship between the speed of impact and the likelihood that the individual struck would suffer an injury. The greater the speed the more likely an injury will be generated.

[73] He knew that in at least 50 to 60 percent of chronic whiplash injuries, the facet joint is involved. Japanese research showed that the facet joint can be crushed with the mechanics of these types of injuries.

[74] People in high-speed collisions are less likely to get better if they have an initial injury than those in lower speed collisions. People in rear-end collisions are less likely to improve than people who have been in other types of high-speed collisions.

Contacts with Hornick:

[75] He saw Hornick on February 10, 2000 at the London Health Sciences Center, University Campus. He reviewed the history and eventually reviewed the medical reports, both pre and post accident, psychological reports, CT scan, bone scan and the SPECT scan of Dr. Reid. Present health complaints were noted and his psychosocial history was reviewed as well as his past medical history.

[76] Hornick was seen again on October 1, 2002. He told the doctor that his pain hadn't really changed. He was still experiencing a lot of left shoulder and neck pain that was extending into some involvement with his left hand. He did not know what the extension meant but said that it was not uncommon for patients who were getting a lot of left-sided pain to intermittently complain of some left hand numbness or some left hand involvement that tended to come and go. Physiologically, one explanation was that the muscles tighten up in the neck and press on the nerves as they go through the brachial plexus. That was called a thoracic outlet type of syndrome. He made no mention of the left hand problem being potentially caused by right frontal lobe brain injury.

[77] The doctor stated that his "assessment overall has not changed. He continues to suffer from a severe whiplash type injury as a result of motor vehicle accident on December 17, 1997 from which he also has ongoing problems with his low back pain. He also has left shoulder pain. It is my opinion that these are soft tissue injuries without indications for bony trauma. Mr. Hornick's prognosis is increasingly poor, as he does not appear to be able to make the adjustment to more sedentary type work. It is a note that Whitney will have difficulty with even the sedentary work because of difficulty with prolonged sitting or standing. Mr. Hornick's options are limited. He either engages in a nonphysical type of employment or he will not be successful with regard to returning to work. He is suffering from severe adjustment disorder which is adding to his problems, significant anxiety, depression and frustration.... continued attempts at counseling to help deal with the adjustment disorder is absolutely critical if he is going to have any potential for success in terms of returning back to work".

[78] Hornick did mention how his life had changed quite dramatically because of the accident. Dr. Teasell felt that it was important to understand how pain affected people in terms of their life in order to try and deal with some of those issues. He was not able to play sports or engage in the same number of family activities. His marriage had broken up

and his relationship with his wife was increasingly estranged. He noted it wasn't helped by his own bad moods and trouble with anger. He was nervous, had panic attacks and was having difficulty with depression. The Dr. felt this could be attributed to the pain and the physical restrictions it caused. He was not sure all of it could be attributed to pain but certainly a large portion of it could be.

Observations:

[79] Hornick had a full range of motion in both knees. There was no evidence of instability of the knee or a tear of the anterior cruciate ligament or the medial collateral ligament. There didn't appear to be any internal dysfunction in his knee but there was tenderness over the medial joint line inside of the knee and in the medial fat pad which resulted in a diagnosis of medial retinaculum. This problem is common and is seen in situations where people's knees strike something like a dashboard or where people have fallen hard on a kneecap. This was a consistent finding because the doctor understood that Hornick had struck his knees up against the dashboard. He had seen about 100 dashboard knees in his practice over the years. They present with very severe swelling and some are severely disabled by just the knee pain while others present with some local tenderness in that area only.

[80] Hornick told him that he had previously hurt his knee but it was a long time ago. Dr. Teasell indicated there was always a concern that, if somebody had a previous injury of the right knee and now were complaining of right knee pain, they may have been more susceptible to developing the pain as a result of the prior injury.

[81] The back injury of 1992 was discussed, the medical reports were reviewed as well as the physiotherapy discharge report of September 28, 1992 which noted that "he had very little complaints, occasional stiffness and a full range of motion". Dr. Teasell opined that if there was a past history of back pain and the person had been in a motor vehicle accident, the person may have been more susceptible to developing back pain as a result of the motor vehicle accident and, there may have been some potential predisposition. He felt on a statistical basis, Hornick was at higher risk of developing problems as a result of the accident than somebody who would not have had those problems beforehand.

[82] Another CT scan of the head was done May of '98 but it showed no abnormality of the brain and no brain injury.

Commenting on other doctor's opinions:

[83] Hornick's relationships with Dr. Yovanovich were reviewed and in particular his report of May 21, 1985 that commented on the twisting injury to his knee some two years earlier [sic 1983]. Dr. Yovanovich had noted that Hornick had improved somewhat with physiotherapy but that "there was still exquisite tenderness over the medial joint line on his right knee. He reported that his right knee had never been a right since his first injury two years previously" [sic 1983].

[84] Dr. Teasell was aware that Dr. Yovanovich was of a different opinion with regard to the knee post accident. Dr. Yovanovich thought the pain was probably due to a combination of the pre-existing meniscus injury described in 1985 and the blow to the kneecap in the accident. He attributed the majority of Hornick's pain to the pre-existing injury that for some reason had flared up after the car accident. He thought he might be able to go in and do arthroscopic surgery to help that knee out. Dr. Teasell disagreed and was not in accord with that analysis. He would not have sent Hornick for arthroscopic surgery.

[85] Dr. Clifford was a colleague of Dr. Teasells who had assessed Hornick in an independent medical examination on June the 10th, 1998. He reported that Hornick had actually sustained, as a result of the motor vehicle accident, predominantly soft tissue injuries to the neck, lower back and left knee. He was of the opinion that the usual healing time for those was 10 to 12 weeks. Dr. Teasell felt that was true of most cases as 50 % of patients with a whiplash injury heal within 12 weeks, another 30 percent will heal over the ensuing two years and 10 to 20 percent would be left with chronic symptoms.

[86] Dr. Clifford felt there was no clear organic based causal relationship between ongoing complaints of pain and remote soft tissue injuries. As well, there was no objective reason why Hornick could not go back to his previous job.

[87] Soft tissue injuries are one of the most common reasons why people aren't able to go back to work. Dr. Clifford felt that the soft tissue injuries didn't provide any objective medical basis for ongoing physical vocational restrictions. Dr. Teasell interpreted that to mean that Dr. Clifford could not find, based on the soft tissue injuries, an objective reason why Whitney couldn't go back to his previous job and he agreed with this analysis.

[88] However, he disagreed with Dr. Clifford's opinion that Hornick was deconditioned and that a 4-6 week program of active therapeutic exercise would result in Hornick being able to return to his usual duties on a graduated basis over the ensuing two to four weeks. He felt that this diagnosis was unrealistic and overoptimistic.

[89] Dr. Clifford felt that Hornick's examination was characterized by perceptions of pain behaviours that undermined the reliability/consistency of the physical examination making it more difficult to draw valid conclusions. In other words, that Hornick may be a hypochondriac or believed what others told him about his injuries or had perceived or anticipated pain. His report seemed to indicate that he felt Hornick was misleading him about his pain. Dr. Teasell disagreed.

[90] Dr. Clifford reported that he had reviewed a surveillance tape where Hornick appeared to move freely during the activities. He didn't feel the movements on the videotape were consistent with the limitations Hornick demonstrated at the time of consultation. He reiterated his opinion that Hornick could go back to his previous physical and vocational activity. Dr Teasell disagreed with those conclusions of Dr. Clifford.

[91] Hornick was seen by Dr. Allen, an orthopaedic surgeon in September after the accident. He felt Hornick had a neck whiplash injury which he described as a chronic soft tissue sprain plus a problem with his knee that may be a small meniscal tear. He did not feel Hornick was capable of returning to the work that Hornick had described he was doing prior to being injured. There was not anything else he could do for him.

[92] He reviewed Clare Latimer's functional assessment and determined the most important part of that assessment was that she identified that Hornick's tolerance or endurance was poor for resistive or repetitive activities involving his upper and lower extremities. This confirmed what Dr. Teasell would have expected with his type of injuries.

[93] Dr. Schnurr, a clinical psychologist, saw him on March 8th and 16th of 1999. He noted Hornick was suffering from problems with pain, but also noted he was having some emotional difficulties with anger, irritability and a mild driving anxiety disorder. He was experiencing unresolved pain. Dr. Teasell felt Dr. Schnurr observed that Hornick was experiencing injuries that had not improved or gotten better and were interfering with his ability to work and enjoy his life to the same extent as he had prior to the accident. Dr. Schnurr commented that Hornick didn't feel that his anger was out of control or in need of treatment. Hornick had some memory and concentration problems but was not in need of psychological treatment because he seemed to be managing as best he could. He felt that he needed alternative vocational plans if he was unable to return to his job at that time. Dr. Teasell felt this confirmed some of the anticipated emotional difficulties Hornick would experience, particularly frustration, if he was unable to perform at the levels he did prior to being injured.

[94] He read Dr. Yvonne Archibald's neuropsychological report. It didn't really factor it into his opinion because he didn't feel he was the best person to talk about mild brain injuries.

Dr. Teasell's assessment and diagnosis:

[95] His assessment and diagnosis indicated that it was most probable that Hornick had suffered a classical whiplash type injury involving the left C2-3 facet joint and that was the most probable primary pain generator in his case. There was some sort of left shoulder strain which would probably be best described as a rotator cuff type injury but he was not sure any injury had occurred at the time of the accident. That type of injury is seen in a lot in patients who had whiplash injuries. He also suffered from a dashboard knee injury.

[96] The prognosis was that Hornick's "pain and limitations will continue, given the amount of time that has past since his accident and the nature of his injuries."

[97] The whiplash injury impaired the functioning of Hornick's body because patients have difficulty with repetitive type work particularly if they have to repeatedly work over their shoulders, pushing, pulling or doing heavy lifting. These activities would exacerbate

or worsen the symptoms. They would only be able to do those kinds of activities for brief periods of time before they got into trouble. They also have difficulty sitting or standing in one position for an extended period of time, particularly if the neck is down in a flexed position, like the chin being down. They don't tend to do well or tolerate jobs where they are sitting with their neck down all day.

[98] He diagnosed mechanical low back strain. The difficulty with a low back pain diagnosis was that in 85 to 90 percent of cases the doctor never knows what is causing the pain because it could be the disc or a facet joint. So the back hurts when one does things but the doctors don't know why and can't determine what structure is the cause, whether it's the disc, the facet, the muscles or the ligaments. He doesn't know that anybody could tell.

[99] Back injuries also cause people to have trouble if they have to do a lot of repeated bending and twisting, heavy lifting as well as pushing and pulling particularly if they do a lot of repeated bending and twisting at the waist.

[100] Dashboard knee is that tenderness around the lower part of the kneecap seen when people strike their knees up against the dashboard. It is a muscle ligamentous problem and is very disabling for people. He has treated it a number of ways including high dose narcotics, anti-seizure drugs and the best antineuropathic pain drugs but nothing has really relieved the pain...

[101] A knee injury like Hornick's would cause him some trouble if he had to do repeated squatting or had to kneel on that knee for an extended period of time.

[102] He was also suffering from "adjustment disorder" which occurs when patients are having trouble adjusting to their pain. Highly active people who are used to being on the go suddenly can't. They get stressed out and the resulting adjustment disorders tend to be characterized by high anxiety, a feeling of not getting things done like before and a lot of anger because they are in pain. They get frustrated and depressed because they realize they have suffered losses. They feel like they can't do all the things they used to do and they are not who they used to be. He thought Hornick was having a hard time adjusting and adapting to this injury even after five years and he has many elements of an adjustment disorder. He had seen patients ten years after an injury who still had not adjusted. He felt Hornick was at his plateau.

Concerning Hornick tricking or misleading him about his physical injuries:

[103] It was possible that Hornick was misleading or tricking him. It was unlikely because the areas where Hornick reported pain were typical of what was seen with other whiplash patients. He felt he would be able to detect deceit but a very clever smart patient could potentially trick a doctor. His experience in having seen a lot of whiplash patients would make it difficult but not impossible to trick or mislead him. He did not believe he was being tricked or misled.

Dr. Teasell's Opinion:

[104] He had referred to background material that had been sent to him in formulating his opinion. Hornick had showed a small disc herniation at C5-6 and some disc bulging at C4-5 as well as some minimal wear and tear changes or degenerative changes at C5-6. It was difficult to know what those changes meant because if a large number of randomly selected people were to have a CT scan, probably half of them would have a similar CT scan even if they didn't have neck pain. More often than not the CT scan is actually fully normal in the patients who have ongoing chronic whiplash pain.

[105] It was his opinion that "individuals with injuries such as these have difficulty with repetitive type work, particularly working with their arms above the level of the shoulder, pushing and pulling or heavy lifting. These restrictions all lead to the exacerbation of symptoms. In addition, they have difficulty with sitting or standing in one position for prolonged periods of time. Mr. Hornick will also have some problems with repetitive bending or twisting at the waist, secondary to back pain"... "it is a note that Mr. Hornick's situation was complicated by the fact that he has always been a hard worker, a very busy individual. As such, it would be anticipated that he would suffer from significant anxiety, irritability and anger, as well as depression as a result of the limitations imposed by the pain". He felt that the key to managing his pain and overall condition would be "proper pacing of activities and getting him into a job which doesn't require him to be continually aggravating his pain. If he can do that, then it is my opinion that he has a good chance of reentering the work force in a physical capacity which he can manage.... there is a high probability of disability in this case, given Mr. Hornick's particular skills and aptitudes, since so much hinges on his ability to adjust, adapt and retrain".

[106] Regarding whether Hornick's physical impairments would have an impact on his pre-accident occupation as a construction type worker at Millers' it was his opinion that people with his type of injuries are not able to engage in that type of work on any sort of consistent basis. He would experience pain that would escalate out of control.

[107] His opinion on the duration of Hornick's injuries was assisted by two studies, one from the U.K. that was ten years old and one from Denmark that was recent and which looked at patients 15 years later in time. Patients were followed for up to two years and then they went on their way. The researchers then found them 15 years later in order to determine how they were doing. The researchers found these patients were in much the same condition at 15 years as they were at two years. People tended to learn that doctors could not help them any further and they moved on with their lives. Hornick was at about the five-year mark and he said "what you see is what you get." There was nothing he could recommend in good conscience that would help Hornick improve from these impairments.

[108] It was his opinion that Hornick had suffered a permanent serious impairment of an important physical function. The primary impairment was his neck

problem, the C2-3 facet injury and, in his opinion, it was serious because it was keeping him from working. It would be permanent because he felt Hornick would not get better. The mechanical low back pain had been apparent for five years. It would persist and will become permanent. It was less serious than the neck pain and, if he didn't have the neck pain, he might be able to get past the back pain. However, the combination of the two make it even more difficult. As well, the knee pain will be permanent and has a serious component. He limps because of it, but he did not think that would necessarily keep Hornick from working. It was his opinion that it was the neck and the back that were keeping him from being employed.

[109] His opinion would not differ if there had been no head restraint. It would increase his risk of injury and would result in a more severe injury. He said the current thinking was that the initial injury occurred on the first snap back and the seatbelt was really only useful on the second coming back forward. In addition, the fact that his head didn't go through the back window did not preclude him from still having chronic symptoms or a chronic neck injury.

[110] At the same time as Dr. Allen, the orthopaedic surgeon, determined there was nothing further that he could do, Dr. Teasell saw Hornick again on October 10, 2002. He concluded that "my assessment overall of Mr. Hornick has not changed. He continues to suffer from a severe whiplash type injury as a result of the motor vehicle accident on December 17, 1997 from which he also has ongoing problems with low back pain. He also has left shoulder pain. It is my opinion that these are soft tissue injuries without indication for bony trauma."..."Hornick's prognosis is increasingly poor, as he does not appear to be able to make the adjustment to more sedentary type work. It is of note that Whitney will have difficulty with even sedentary work because of difficulty with prolonged sitting or standing. Mr. Hornick's options are limited. He either engages in non-physical type employment or he will not be successful with regard to return to work. He is suffering from a severe adjustment disorder which is adding to his problems, significant anxiety, depression and frustration."

The videotapes:

[111] He reviewed three videotapes of Hornick and was of the view that "there's nothing here that is inconsistent with this gentleman's physical findings at the time of my examination when he had full range of motion, nor are there any inconsistencies with his self-reports of activity. At one point he shovels snow for short periods of time; we do not know what the knee experiences afterwards. The amount of activity seen on the tape is not sufficient to say he can return to work as a road repairman. This does not in anyway change my opinion. It's a note that he seems to walk with a slight right -- sided limp".

[112] Someone with a whiplash can bend over and shovel away some snow while he was bent over. Hornick's actions appeared to be awkward. He seemed to be moving small amounts of snow. Later at 11:12 he moved some more snow around but did it with

more effort. Each shoveling episode lasted about two to three minutes. He seemed to want to shovel his driveway but he also seemed not to want to do it all at once. His impression was that Hornick could handle the shoveling for a short period of time and then he would try to finish it off later after he had had a chance to rest. The videotape did not alter his ultimate diagnosis or opinion in any way. I agree with these observations.

Dr. Hoffman:

[113] He was a duly qualified psychiatrist with expertise in the emotional consequences of personal injury with emphasis on closed head injuries. He had written a handbook called *Emotional Consequences of Personal Injury: A Handbook for Psychiatrists and Lawyers* with the latest edition released in 2001. He had testified in over 1000 civil litigation cases approximately balanced between consulting for the plaintiff and the defence.

[114] He reviewed many medical, physiotherapy, psychological and neuropsychological reports concerning Hornick before he came to his conclusions. He saw the plaintiff for two and one quarter hours on May 7, 2002, some four and one half years after the crash. He reviewed pictures of the crash and a medical brief that was forwarded to him by counsel for the plaintiff.

[115] Hornick related the events on the day of the crash that was more or less consistent with the evidence he gave during the trial. He said he was "sore and stiff" all over including the back of his head, neck, left shoulder, left elbow, lower back, right knee and both heels. He was dizzy and somewhat confused immediately following the crash.

[116] He reported that he had not gotten much better since the accident but continued to suffer from soreness and stiffness throughout his body as well as dizziness and poor balance. As well, he felt unsteady and tended to fall off to the right side if he got up to quickly. He continued to have pain in the region of his lower back and neck. His lower back was aggravated by bending, twisting or walking more than a quarter mile. Back pain improved with stretching exercises. Neck pain was aggravated by turning his head from side to side or by making repetitive head movements. He also described decreased memory and concentration difficulties.

[117] The argument with his wife, the connection with the police and the fact that he and his wife never reconciled after the incident were also discussed. His anger and frustration because he was out of work and separated from his wife continued.

[118] He reported that his marriage fell apart after the accident because his wages stopped and the insurance money did not "kick in" and so he had no money. He described feeling sad, anxious and angry which he related to financial difficulties caused by the system, the marital breakup and his separation from his sons.

[119] He was depressed, suffered weight-loss, insomnia, agitation, fatigue and poor concentration. He said his depression was worse when "the system jerks me around".

[120] He told Dr. Hoffman that Drs. Sweeney and Archibald had told him he had a closed head injury.

[121] The doctor commented on the fact that Hornick refused all treatment offered by the ambulance driver and appeared confused at the hospital. It was his view that this was irrational behavior and one of the signs of concussion as a result of head and brain injury.

Dr. Hoffman's opinions:

[122] The clinical evidence suggested, on a balance of probabilities, that Hornick did suffer a brain injury to the frontal lobe of the brain. Further, he accepted the fact that the plaintiff's wife reported that her husband did not have cognitive, emotional or behavioral difficulties prior to the motor vehicle accident.

[123] In his report he stated "there is documentation that Mr. Hornick suffered amnesia following the accident. The ambulance attendants describe Mr. Hornick as confused and recorded a GCS of 12. The patient's symptoms of anger and irritability are typical of frontal lobe syndromes. The Spect scan of the brain supports a diagnosis of a brain injury in an area of the brain that is susceptible to damage in acceleration - deceleration injuries, and would account for changes and Mr. Hornick's behavior". Further, "the neuropsychological and psychological testing of Drs. Archibald, Casey, Sweeney, Leenaars and Smith supported the clinical diagnosis of brain injury". He felt that the psychological testing, by and large did not support a diagnosis of malingering.

[124] His provisional psychiatric diagnosis was "personality change due to head trauma". He also suffered from "chronic pain disorder associated with both general medical conditions and psychological factors".

[125] He agreed with other examiners who felt that "Mr. Hornick is trying to convey his sense of desperation because he has not felt validated or believed within the adversarial insurance system".

[126] He felt that it was unlikely there would be any further improvement in Hornick's cognitive or pain symptoms and it was unlikely that he would become competitively employable.

[127] He was satisfied that Hornick had sustained a **permanent mental impairment** because he had a loss of executive functioning which concerned his ability to do multi-tasking, to plan ahead and to do complex thinking. He had lost the ability to maintain his self-esteem which most people take for granted. As well, he had lost the ability to maintain his mood in a calm and relaxed manner.

[128] Regarding whether Whitney Hornick had been suffered a **permanent impairment of an important psychological function** as distinct from a mental function the doctor indicated that “I’m afraid I cannot distinguish mental from psychological. I recognize the law includes both, but as a psychiatrist I don’t know the difference between mental function and psychological function”.

[129] He agreed that the above opinion would cover both the mental and psychological functions.

[130] Regarding whether he could foresee any change in the future in these impairments and whether the plaintiff will improve in the foreseeable future, the doctor opined that “they are unlikely to improve in the foreseeable future. The reason given was because “in emotional or psychiatric states, the longer you get caught up in emotional suffering, then the longer it continues. We don’t know whether that is because a person develops the habit of being there, of just suffering, being anxious, depressed and angry or whether it’s because the environment changes and you lose supports the longer you stay there or, whether in fact such states even produce a change in the transmitters of the brain that make it difficult to overcome”...” Our moods, every thought must have some chemical change in the brain for which we have absolutely no explanation, but the longer a person gets caught up in psychopathology the more likely it is to continue”.

[131] He was not asked and did not deal with the topics of the seriousness and importance of the impairment.

Margaret Bertoia:

[132] She was a registered physiotherapist who she saw Hornick first on January 6, 1998 within 30 days of the crash. This was important timing because Dr. Teasell said if the right knee injury had not been reported within 30 days it may not be crash related but if reported within 30 days it could be crash related. She reported “On examination, at initial assessment and at reassessments since then, Mr. Hornick has presented with decreased...” range of motion “...of the cervical spine, lumbar spine, poor posture, decreased flexibility of his hamstring”... “straight leg raise on (L), swelling in right knee (+) McMurrays test for pain only, tenderness in cervical spine, lumbar spine, bilat. heels, medial aspect of right knee.”. Further, she reported that “his main complaints at latest assessment including pain in the right knee, headaches, and feels that he “head is not right” — referring to his vision, balance, dizziness he continues to experience.”

[133] It was clear Hornick had reported having difficulty with the right knee well within the 30 day time frame and having some difficulty in terms of having a possible brain injury.

Dr Patel:

[134] He was a duly qualified and licensed general medical practitioner who had been Hornick's family doctor since the 1973. He was a very busy rural family practitioner. His pre crash notes indicated as follows:

Right knee:

[135] On April 22, 1985 he noted that Hornick had a right knee injury of two years duration. He was never told by Hornick that he had had a hockey injury in 1983 and that he was off work for six weeks. He was referred Hornick to Dr. Yovanovich, an orthopedic specialist on May 21, 1985. Dr. Yovanovich reported on April 25, 1985 and Dr. Patel noted that his findings were rather nonspecific but the complaints suggested a meniscal tear which was likely reinjured during the week of April 18, 1985.

Back:

[136] On December 9, 1991, he indicated that Hornick told him he was getting water out of the pool the day before and developed a sharp pain in the right lower back. On examination, he was tender over the right S1 joint. He was still tender in the L5 -- S1 area and in pain on December 13. On examination on December 16, it was noted that Hornick felt better but there was some discomfort at his side and he had a slight scoliosis to the left.

[137] Hornick was lifting a freezer at his house on May 19, 1992 and developed severe pain in his back. He was tender over the L5 -- S1 area with paravertebral muscle spasm. There was slight improvement through May 25 and on May 29 he was referred to physiotherapy with Dr. Halpenny. Dr. Patel noted that by October 2, 1992 Hornick felt better, had finished physiotherapy and had full range of motion in lumbosacral spine. He was to return to work on October 5, 1992.

[138] Those incidents set out in detail the complaints of Hornick with respect to his knee and back to 1992. There were no complaints with respect to either area between 1992 and the crash on December 17, 1997.

[139] In addition there was no indication up to this point in Dr. Patel's notes that Hornick required anger management or psychological counseling.

[140] His notes indicated very few examples of blood pressure elevation. They occurred after Hornick complained to the doctor about certain health events. The findings were not consistently noted, no medication was prescribed and there was no indication of a blood pressure problem.

Contacts with Whitney Hornick from December 17, 1997 to date:

[141] He saw Hornick on December 18, 1997. He related some details of the accident including the fact that he could not remember the impact. He complained of pain

in the neck, back, left elbow, left hand and left knee. He did not mention any pain in his right knee. He had limitations a movement in his neck. There was swelling and tenderness in his back muscles and over the patella. On December 29 he requisitioned rehabilitative physiotherapy. On December 30, he complained of pain in his neck and upper back. On January 13, 1998 he was still in pain, receiving physiotherapy and was having difficulty with the cervical spine and paravertebral muscles in his back. Dr. Patel was unable to say whether Hornick had reinjured his back [the 1992 injury].

[142] He booked a CT scan because of these headaches and the pain in the neck. The scan showed no abnormalities. It was his opinion that, notwithstanding a negative CT scan, the patient can still have pain.

[143] On February 10, 1998 he felt that his right knee was swollen and it was his opinion at that date that Whitney would be unable to do modified work because of his neck and back pain.

[144] Dr. Patel recorded that Whitney was seen by Dr. Fleming on February 27th 1998. He was told at that time that he had a bad whiplash injury to his neck. There were still some slight paravertebral muscle spasms and his spine was tender in the lumbar and sacral region. He was still getting physiotherapy every second day. Throughout the remainder of March, April and May of 1998 the same types of complaints were being made with respect to his neck and back as well as complaints of occasional headache, dizziness and blurred vision.

[145] On May 26, 1998, Dr. Patel noted that Hornick was quite depressed, unable to sleep and had to go for walks in order to get the stress relieved.

[146] His complaints continued and on August 6, 1999, he indicated that he still had pain in his neck, headaches, limitation of motion in the cervical spine and paravertebral muscle difficulties. Hornick continued to be depressed and anxious because of the constant vigilance by the insurance company.

[147] As of December 7, 1999, he felt Hornick had not improved and continued to have headaches, back and neck pain. Hornick had followed all of his recommendations in terms of treatment.

[148] September 21, 2000 Hornick indicated that he was having marked episodes of forgetfulness because of "closed head injury". Hornick used those words and indicated that he had been told by doctors that he had suffered a closed head injury.

[149] By December 21, 2000, three years after the motor vehicle accident, Whitney showed no improvement and was suffering from the same difficulties which the doctor described as chronic pain.

[150] On December 6, 2001, almost four years after the accident, Hornick climbed up a ladder to put up a chandelier for his mother, got dizzy, fell and injured his right knee, ankle and right shin. Dr. Patel felt that there had been no improvement in his overall physical condition and that Whitney was still suffering depression. His observations did not change up to and including the last time he saw Hornick on July 29, 2002.

[151] At the end of his evidence in chief, Dr. Patel said that Hornick was still following all of his recommendations but that there was no other treatment that would assist him. It was his view that nothing yet had cured Hornick's problems.

[152] His early notes are very important in determining if there was collateral causation for the injuries to his knee and back. In addition, there was the evidence of his co-workers who made no mention of Hornick not being able to do his work or that he complained of knee or back pain before the crash.

Dr. Allen:

[153] He was an orthopaedic surgeon who saw Hornick for almost 4 years but had died tragically in the air plain crash before the trial. He saw Hornick and issued a report on September 23rd, 1998 almost a year after the crash and stated at pages eight and nine.

"The diagnosis at the present time as far as the neck is concerned is of a whiplash injury or chronic soft tissue strain." [similar finding as Dr. Teasell] "that of the knee there is a possible intra-articular derangement, such as a small meniscal tear and that of the low back is of a soft tissue strain or mechanical low back pain. This patient continues to have significant discomfort and pain with regards.." to "...these injuries. It is difficult to determine how much of a role if any, that a chronic pain syndrome may play here. However, one would expect the soft tissue components of these injuries to have been healed by the 6-12 week mark. Although ongoing pain is certainly very common after these injuries for up to a 2 year mark. It is very difficult to give any significant prognosis at the present time, I think that it is somewhat premature. However, I suspect that it is quite likely that this gentleman will continue to have some significant ongoing problems which may well be combined physical, as well as psychological problems because of this prolonged pain and it seems quite likely that this will prevent him from returning to the type of work as described above. I do not feel personally, that this gentleman is capable of returning to the work that is described to me."

[154] In his last report of October 3rd, 2002, he stated “at the present time, I don’t think anything has changed from his previous examination. I don’t think my assessment would change either. I still feel that he is probably unfit for any gainful employment and likely will remain so forever, if not at least a very extended period. Apart from an arthroscopic examination of this right knee, I can’t think of anything surgically that would be of any significant benefit to him...I believe these injuries for the most part are directly related to the motor vehicle accident that he suffered in December of 1997 and at present, I don’t have any further to offer him treatment wise unfortunately.” He clearly concludes that he cannot help Hornick.

Dr. Archibald:

[155] She was a clinical neuropsychologist who saw Hornick March 11, 1999 one year and 3 months after the crash. She was the first neuropsychologist to see Hornick . On page 6 under the heading “Summary, Conclusions and Recommendations” she wrote

“This 40 year-old man suffered soft tissue injuries in a motor vehicle accident on December 17, 1997. The question also arises as to whether or not he sustained a traumatic brain injury. Data from the neuropsychological assessment on March 11, 1999, 15 months following the accident indicate a number of cognitive difficulties. 1. Reduced working memory for verbal tasks which serves to reduce the Verbal I.Q. from Low Average to the Borderline range of functioning. 2. Poor short-term memory and slow speed of processing information. 3. Difficulty with the registration of visual material, including faces, family scenes and a complex drawing. 4. Below average executive functions. Reduced mental fluency and reduced mental flexibility with some difficulty formulating concepts on a problem-solving task were documented. Planning of a complex drawing was also poor. 5. Reduced grip strength in the preferred left arm, associated with pain in the left shoulder. Slow manual dexterity in both hands and somewhat slow speed of finger tapping were also documented. 6. A high level of emotional distress.

She goes on at page seven,

“Mr. Hornick’s Glasgow Coma Scale, measured by the ambulance crew was 12, consistent with mild traumatic brain injury. He was confused for a short, undetermined period of time. This confusion had cleared by the time he was assessed in Emergency. There was no evidence of a focal neurological deficit upon examination at the hospital. Mr. Hornick does not appear to suffer from retrograde amnesia, but does appear to have suffered a short period of loss of consciousness and post-traumatic amnesia, and was confused at the scene. It is probable, therefore, that he sustained a mild traumatic brain injury. The force with which his truck was hit would be consistent with an acceleration/deceleration type injury. He may also have sustained a diffuse axonal injury from shearing forces applied through the brain. Such injury can usually only be detected microscopically. “Mild

traumatic brain injury is frequently followed by a post-concussive syndrome. Symptoms include poor concentration and memory, headaches, sleep disturbance, hypersensitivities, neck pain, anxiety and irritability. On neuropsychological assessment working memory is reduced, information processing is slow, and memory/learning and executive functions may appear limited. Mr. Hornick's physical and neuropsychological / psychological presentation is consistent with a post-concussive syndrome."

She goes on to say,

"In Mr. Hornick's case, it is probable that he sustained a mild traumatic brain injury in the motor vehicle accident of December 17, 1997. Neuropsychological signs typically resolve by the one-year mark. Their persistence in Mr. Hornick's case is likely attributable to additional contribution factors such as headache, pain, reduced energy, poor sleep and emotional distress. I should also point out that there is a small subgroup of individuals in who brain injury results in neuronal dysfunction that leads to persistent symptoms. There is a possibility that Mr. Hornick falls into this group."

Dr. Sweeney:

[156] This doctor was a specialist in psychology with an emphasis on neuropsychology. He both assessed and treated Hornick for an extensive period of time. He spent a total of approximately 47 hours dealing with this case on behalf of the Plaintiff and was criticized by defence counsel for the time spent and was accused of being an advocate for the Plaintiff. When all was said and done I do not agree he was an advocate. He did a very careful assessment of the situation, did the appropriate tests, may have been a little confused on one date which did not affect the credibility, believability and reliability of his evidence and also treated Hornick which gave him a more complete sense of what the situation was all about from a psychological point of view.

[157] It was his opinion that the plaintiff did suffer a brain injury and that he was "moderately impaired especially in regard to problem-solving, which involves abstract reasoning and concept formation, and psychomotor function, thinking while doing. Those two areas are the most salient areas of deficiency".

[158] Secondly, "he could not have had these impairments when he was gainfully employed because the nature of his employment required him to draw upon these skills and he did so successfully.... as a supervisor of a group of people, as a group leader in a furnace factory. He managed inventory when he helped operate a service station, so he did demonstrate abstract reasoning and organizational ability and psychomotor function". He

went on to say that the plaintiff "would not have been able to repair, restore and perhaps sell used cars. He would not have been able to be involved in that kind of the business because he did that sort of thing from the age of 18. It involved a lot of problem-solving and he would not have been able to do that had he been impaired prior to the accident". So, "the historical information and the clinical data that I obtained support and ideology of head injury in the motor vehicle accident."

[159] He was then asked whether the plaintiff had suffered a mental impairment and he indicated that he interpreted the words "mental impairment" as including cognitive impairment, emotional dysfunction and psychological difficulties. It was his view that the plaintiff had all of those. As a result the doctor felt that Hornick was significantly and seriously impaired mentally.

[160] He was asked whether he had an opinion as to whether the plaintiff had suffered a psychological impairment. It was his view that a psychological impairment was included in a mental impairment because the words mental impairment were more general. The words "psychological impairment" include "neuropsychological impairment", because neuropsychology comes from psychology. It was his opinion that the plaintiff did have a psychological impairment because he was moderately impaired neuropsychologically. Also, he had an emotional control problem and that too is a psychological impairment. In the end it was his opinion that the plaintiff has a significant and serious psychological impairment.

[161] Regarding the question as to whether the mental and psychological impairments were important to the day-to-day function of Hornick, it was his view that these cognitive difficulties were going to "seriously diminish the quality of this man's life". "He is not going to be able to understand the world around him as well as he could before. Certainly, he is not going to be able to function in employment in a managerial capacity which demands problem-solving". His employment options are seriously restricted as he is going to have difficulty reacting emotionally to the everyday irritants of daily life. He will find himself emotionally upset when most people aren't. It will preoccupy him and interfere with his cognition. Even when he is stable emotionally, he will have difficulty figuring things out and knowing what's happening around him."..." the poor man is quite disabled in terms of day-to-day function in a lot of areas, cognitive, psychological, and emotional" and, it is not going to go away.

[162] He was asked whether these mental and psychological impairments to important bodily functions were permanent. He saw Hornick for years after the accident. He would have recovered all that he was going to recover by that time so, "I definitely would conclude that these neuropsychological impairments are permanent, and that's the way Whitney is going to remain for the rest of his life." He recommended that a Spect scan be done which was done later by Dr. Reid.

[163] His report after the second neuropsychological testing indicated that:

“Considering that Mr. Hornick (1) suffered direct impact to the back of his head in the motor vehicle accident, (2) had a Glasgow Coma Scale of 12 and appeared confused following the mishap, (3) was exposed to acceleration forces in the current accident violent enough to produce an extended period of post-traumatic amnesia, (4) reported the post-accident onset of a profile of cognitive complaints consistent with present test results, (5) provided test results characterized by a limited number of clear deficits, mainly implicating one cerebral hemisphere and (6) yielded clinical data compatible with the results of published neuropsychological research studies regarding nonimpact brain injury conducted in our laboratory, I would conclude that this man is suffering the ongoing debilitating effects of significantly diminished neuropsychological status as a consequence of head injury in the motor vehicle accident of December 17, 1997.”

[164] Regarding the validity of Dr. Shapiro's evidence, he felt that her data was invalid because Hornick had not been properly prepared by her and the psychometrist before he was tested. Further, it was apparent that she felt that the Glasgow Coma Scale was 15 at the scene when fact it was 12. It was his view that this might have caused Dr. Shapiro to go back and look at her information and maybe conclude that her opinion was not right. In other words, she may well believe that it was less likely that there was a neuropsychological impairment. Its presence would be less likely. However, even if the Glasgow Coma Scale was 15, a person can still suffer neuropsychological impairment. The Glasgow Coma Scale reading of 12, after full testing, would lead Dr. Sweeney to reevaluate his data because that number is an indication that there is an increase in likelihood of brain injury.

[165] He was asked to comment on Dr. Shapiro's belief that Hornick was a person who may well intentionally knowingly suppress a correct response. He believed that a person who was tested 2 years apart and came up with the exact same extent of impairment was simply not capable of that. The truth was that he was moderately impaired and that had been demonstrated not once but twice, two years apart, by professional competent testing. That established that there was something wrong with Hornick.

[166] He was asked if Hornick was a malingerer. He responded that “the client's cooperative, highly motivated test behaviour, the meaningful clinical profile demonstrated, and the absence of positive psychometric indices of malingering rule out any possibility of exaggeration or outright malingering.”

Dr. Reid:

[167] Dr. Reid was a specialist practicing at University hospital in London Ontario in the Department of diagnostic radiology and nuclear medicine with consulting privileges in nuclear medicine. He has been in that position since July of 1992.

[168] He described what single photon emission computerized tomography or a SPECT scan was all about. It involved the injection of a radio isotopes and other drugs in order to obtain images of organs in the body like the brain.

[169] While CT and MRI scans record anatomical information, the SPECT scan records the subject organ's functionality.

[170] Imaging was performed and sagittal [looking at the right and left parts of the brain from the front], coronal [looking at the frontal plane along the long axis of the body, a side view of the brain] and transaxial [looking at right angles to the long axis of the body] views were reconstructed.

[171] The doctor felt when he did the scan, that the results required an anatomic imaging correlation as well as a comparison with the neuropsych testing because it was his opinion at the time that there was a possible traumatic etiology. He went on to say that correlation with clinical neuropsychological data would be necessary to assist in confirming his opinion. He had not referred to any of that data before he interpreted his findings from the Spect scan.

Opinion:

[172] Subsequent to his testing he did in fact read the information that had been given to him which included the clinical neuropsychological data. He confirmed his opinion there was brain injury and that it was consistent with having been caused by the motor vehicle accident, December 17, 1997. His exact words were in response to the question "so, is the scan consistent with a high-speed rear end impact" and he responded "the distribution in the scan is consistent with an injury from -- what's the word I want -- it would be easy to say yes to your question, but I can't quite get -- I want to qualify and I can't quite get the way I -- it's consistent with an injury from trauma that is -- but I frequently seen in motor vehicle accidents where patients have had a whiplash type of mechanism."..."it's probably -- well, it's probably a whiplash forward to back, but I could say that wasn't -- you know, it's hard not having -- hypothetically speaking, it's probably a whiplash front to back ..." He then added, "well, it can be two ways. Either you're head from the back or you come in contact with an inanimate object from the front..." There was no question that the doctor knew that the plaintiff had been rear ended by truck. He was asked "would you expect to see that [the brain injury] is consistent with a rear end collision where a car moving slowly or stopped gets hit from behind by another car?" And he answered "yes".

[173] He was asked about the impact of the clinical data on his opinion and he replied that "I think I said earlier that I was 70 to 75% sure without the clinical data and the clinical information that you have sort of given me, that basically solidifies that I really can't think of anything else that would give me that pattern of abnormality, because of all the patients that I have seen, probably about 12-1300 of them have probably been

head injuries, and the distribution of abnormality year is surprisingly robust for that type of injury."

[174] By way of explanation, he introduced exhibits 43 "A" and "B" which showed the results of the scan. In the transverse, coronal and sagittal portions of the photographic results, he pointed out the particular areas of traumatic etiology.

[175] Both he and Dr. Soucy [for the defence] used the BRASS software program to run their SPECT scan on patients they were treating. It was the generally accepted standard SPECT analysis program. When Dr. Reid ran his SPECT test on Hornick and set the computer program at the "standard generally accepted mark of two standard deviations from the norm", the green dots appeared. Hornick could not fake or make that up.

[176] I conclude that short of an autopsy on Hornick, the Spect scan was the only piece of objective evidence of brain damage the court had.

[177] The question became, was it reasonable to conclude that the brain inside Hornick's body, that was catapulted around in that truck came away without any brain injury? Or does it make probable sense, on the balance of probabilities, that the brain was injured as well. In support of the "yes" answer was the evidence of Dr. Reid, who did his testing and looked at all of that evidence and then looked at the neuropsychological reports, learned about the violence of the crash and concluded that he was about 70, 75% sure that the crash injured Whitney's brain. Also in support were the opinions of Dr. Hoffman, Dr. Archibald and Dr. Sweeney.

DEFENCE EVIDENCE:

[178] No evidence was called by the defence to contradict the evidence of Hornick's co-workers or his family with the exception of the evidence of Pam concerning the emotional and limited physical abuse she sustained from time to time.

Dr. Shapiro:

[179] She was a duly qualified psychologist licensed to practice in the province of Ontario with a Ph.D. in clinical and counseling psychology. She agreed that her practice was primarily medical legal referrals of which 80 % were for defendants. The majority of her income was derived from forensic neuropsychological testing requests from insurance companies. She confessed that she gave lectures on feigning tactics of plaintiffs to her clients.

Preparation of Hornick to take the tests:

[180] Her interview with Hornick lasted approximately 90 minutes. She had her in house psychometrist do the actual administration of the tests before she saw him.

[181] Hornick indicated he did not know whether he could handle the evaluation because he had done several already. The psychometrist's observations indicated that it was "very difficult to establish and maintain rapport with Mr. Hornick. A working accord was eventually established after about an hour, but Mr. Hornick became quite irritated with any sign of support or encouragement from the examiner. He seemed very suspicious and mistrustful and was quite impatient. He often seemed to be testing the limits and was frequently flippant, curt and sarcastic. He threatened to leave several times". Further, during testing, "Mr. Hornick was cooperative to the extent that he did not refuse to do tasks outright but motivation seemed minimal and effort seemed to vary widely even within a single test. He often disregarded details, and seemed uncomfortable with doing too well, for example, during one test, he commented that 'this is too easy -- according to this, I'm fine'. He aborted some tasks by refusing to continue until the time limits were reached remarking 'that's enough for you'. He seemed easily bored and required a good deal of urging. He was not receptive to guidance. Frustration tolerance was poor and Mr. Hornick used profanity throughout.... he expressed skepticism regarding the purpose of the tests and the assessment in general and complaints throughout.... he maintained a dour and frowning expression and often seemed very angry and irritable". He mumbled when he spoke and at times did not seem to understand the questions and responded with information that had not been requested. Hornick indicated that he was unsure as to whether he could tolerate the assessment. He had difficulty retaining information.

[182] In her report she stated that "according to the ambulance attendants he had a Glasgow coma scale of 15 at the scene.....". She agreed that was not accurate. Simply because the scale was 12 and she missed it, her opinion did not change because the performance still spoke for itself

[183] She acknowledged that she knew of Dr. Margaret Kuiack, a psychologist with a practice in neuropsychology. She also knew that Dr. Kuiack attempted to test Hornick but sent Hornick away because she determined that his motivation was not appropriate for testing. Notwithstanding she knew of Dr. Kuiack's reaction, she ordered the tests to be done and interpreted them. She disagreed that it would have been better to send Hornick away and not do the tests because of his motivational difficulties. Dr. Kuiack's position was echoed by Dr. Sweeney and he too felt the tests should not have been done at that time because of Hornick's lack of motivation to proceed. Any results would be skewed.

[184] Dr. Schapiro acknowledged Dr. Sweeney's view that one of Hornick's problems was that he overreacted emotionally to minor irritants. She agreed that this neuropsychological testing by her was the fifth time Hornick had gone through it and that it was at least a minor irritant to him.

[185] She found that his “intellectual functioning” overall was very low, much lower than normal. His verbal skills were much lower than his nonverbal skills. With respect to “executive functioning” which means being able to analyze situations, make plans adaptively and mental flexibility, the results were mixed. This suggested some inconsistency in performance and suggested that Mr. Hornick might be able to do better than the low scores indicated. Her final opinion with respect to memory and learning was that the results which she saw simply could not be an accurate reflection of cognitive impairment.

[186] The Hooper test dealt with visual spatial matters and he did not do very well at all in that test. Pictures of familiar objects were broken into parts and Hornick was asked to look at the parts and identify what they might be. He had a great deal of difficulty with this particular task. It was her view that these test results were an anomaly because in every other regard the visual spatial functioning appeared to be normal.

[187] The Word memory test measured memory and feigning. His responses led her to believe that Hornick was able to suppress the correct response and provide the incorrect response. In other words, it was her opinion that Hornick was making a deliberate effort to appear more impaired than would actually be the case and that raised questions about the validity of the test scores from all the other test materials that had been obtained.

[188] Dr. Shapiro said that the literature indicated in cases of mild injury, that 35 to 40% of patients who were seeking compensation were likely to feign test results to look more impaired.

[189] Under the heading in her report " Summary and Opinion" she stated that the test results suggested Hornick was of limited intelligence especially with regard to verbal skills and that some of the levels of impairment were severe but didn't make sense from a cognitive standpoint. It was her view that his efforts were designed to underachieve. As a result she felt that the test scores themselves were not meaningful. She felt that Hornick was capable of doing much better than his score suggested.

[190] The test scores suggested a level of impairment that was not supported by the patient's clinical presentation. In terms of his cognitive functioning the test scores were likely the minimal of which he is capable.

[191] As to whether Hornick had any neuropsychological impairment as a result of the motor vehicle accident, in was her opinion that if there was any such impairment, it was very modest and may not be directly attributable to the accident itself. It may have been pre-existing.

[192] Dr. Shapiro was asked to comment on Dr. Sweeney's position that if there were invalid test results, those results were useless in considering neuropsychological impairment. She disagreed totally because the pattern of test scores was instructive and

while the total score may suggest impairment, when one looked at the pattern of response in was evident that there was more skill that was being reflected in the psychometric properties of the total score.

[193] She did take into account the fact that Dr. Reid's Spect scan did show some abnormalities. Those abnormalities could have been old and there was no way to determine dating in her opinion. She believed that Dr. Reid's scan indicated an abnormality in the right frontal lobe of Hornick's brain. She also acknowledged that she had Dr. Sweeney's report where his opinion was that there was a dysfunction in the right frontal lobe of Hornick's brain. Further, she agreed that Dr. Archibald's opinion in her neuropsychological report indicated that she believed Hornick had suffered brain injury as well.

[194] She acknowledged that the brain was divided into two hemispheres, the right and the left. The right hemisphere controlled the left side of the body and the left hemisphere controlled the right side of the body. There were tests that can determine the existence of lesion in a particular hemisphere but she did not specifically employ the use of any psychometric tests that were sensitive to the detection of right left differences in Hornick. Notwithstanding she was aware of Dr. Reid's, Dr. Sweeney's and Dr. Archibald's reports indicating brain trauma, she did not do anything specific to determine right left differences. The Spect scan was an objective test. She agreed that Spect scan abnormalities may have pre-existed the crash but that it was equally possible that the abnormalities may not have existed before the crash and may only have existed after it and as a result of it.

[195] She acknowledged that Dr. Sweeney did do some symptom validity testing but that the specific test that he used was not the most sensitive. Dr. Shapiro did some symptom validity testing as well.

[196] Hornick's counsel asked Dr. Schapiro to assume that before motor vehicle crash Hornick did not forget all kinds of things and did not maintain a written daily diary to maintain appointments. However, after the accident he forgot all kinds of things like his son's sports practices and doctor's appointments. She agreed that this was one indicator that Hornick had suffered brain injury. They were also consistent with physical pain, headache, stress, emotional upset, all of those elements could provide the same symptomology.

[197] Motivation and effort in dealing with the tests that she administered were very important when it came to the validity of the neuropsychological testing. In fact she went so far as to say that it was absolutely critical in order to find valid test results. Minimum effort equals minimum results. Notwithstanding her view that Hornick's efforts were minimal and the results would be invalid or minimal, she said, at the bottom of page 14 and top of page 15 of her report, that "as a result, from a neuropsychological standpoint,

Mr. Hornick does not demonstrate any substantial cognitive impairment that can be directly attributed to the subject motor vehicle accident."

[198] She agreed that from the test results she obtained, she could not assist as to whether Hornick still would be able to perform the work that he did at Duo-Matic.

[199] People with mild brain injury will obtain a very significant recovery within 90 days with subsequent recovery up to and including two years post-trauma. However, not all recover fully. 10 to 15% of victims with mild traumatic brain injury do not fully recover even after two years of the point of trauma. She agreed that Whitney Hornick could fall into that 10 to 15%.

[200] She indicated that she knew Dr. Rourke and that he had an excellent reputation. She was asked to assume the Dr. Rourke had the test scores from Dr. Sweeney and all the medical and psychological reports before he gave his opinion. She was asked whether it would be appropriate in those circumstances for Dr. Rourke to give his opinion and she replied "absolutely." She said it was common practice to review test scores in conjunction with the appropriate reports and the medical file and then proffered opinion.

[201] She did agree that the results of her spatial testing where Hornick had some difficulty copying a complex geometric figure and had great difficulty identifying a series of fragmented objects indicated a very high probability of impairment.

[202] Finally, it was her opinion that the most that can be said was that any cognitive impairment that may exist was likely much less than suggested by these test results. As a result, from a neuropsychological standpoint, Hornick did not demonstrate any substantial cognitive impairment that can be directly attributed to the subject motor vehicle accident.

Dr. Rourke:

[203] He did not see Hornick but reviewed his records especially the neuropsychological reports. He agreed that Hornick exhibited some neuropsychological deficits but "these could very well have antedated the accident". After reviewing the reports he concluded that "most if not all of Hornick's current neuropsychological deficits are probably a reflection of limitations that he experienced prior to the accident in question' and he "is not suffering from any type or degree of neuropsychological deficits that would prevent him for engaging in his previous work". He did not set out any reasoning for his findings that would be of assistance to the court in assessing the weight of his opinion. This evidence was not very helpful at all.

Dr. Clifford:

[204] He said there was no substantial difference in grip strength from left to right hand and he was not cross-examined on that point.

Dr. Soucy:

[205] He too completed a Spect scan like Dr. Reid. It showed a distribution of activity that was slightly diffusely heterogeneous given Hornick's age. Borderline significant reductions of a more focal nature were seen in the cortex of the right orbito-frontal area. Those areas were indeed considered abnormal upon automated comparison of Hornick's study to a reference study. There was also mildly asymmetrical uptake of the tracer by the basal ganglia, with the right side being less active than the left but did not reach significance levels on automated analysis.

[206] The diffuse heterogeneity of uptake and the accompanying local anomalies were all mild and of borderline clinical significance. In particular, "because of the patient's age, and also because of the right-sided hypoperfusion of the basal ganglia, the current study was compatible with, albeit in no way specific for, a past traumatic brain injury (not necessarily that which happened in 1997)".

[207] There was probably a mild anomaly. However, given the limited nature of the anomalies observed, even a pronounced normal variant cannot be ruled out, nor could a number of other types of brain injuries. Brain injuries of different types can result in frontal or temporal lobe anomalies, but there were a variety of other types of lesions or normal variants or technical things that can happen that would give images that would be similar to the kind seen in the study.

[208] One of the causes could be a trauma, but there were other reasons why one could get this kind of image. The lesion could not be dated in any way in terms of its age.

[209] However, given the limited nature of the anomalies observed, even a pronounced normal variant could not be ruled out, nor could a number of other types of brain injuries. What was seen were a series of very mild deficits in a patient who had somewhat heterogeneous activity overall. He was not completely convinced that what he saw was a significant lesion. It could have been a normal variant.

[210] When he read Dr. Reid's report and compared it to his, he was quite pleased to see that two people living a few hundred miles apart could come up with a remarkably similar description of the case. Basically everything he reported was the same, more or less, as what Dr. Reid said in his report. The description of the case was virtually the same. A clinician who received one or the other would have the same information.

[211] He felt the finding was mild enough that it would be hard to determine if there was actually something abnormal or not. If there was something abnormal, it was mild, mild enough that it could be attributed to a non-pathological factor. They basically saw the same things and that was comforting to Dr. Soucy.

[212] He was informed that Hornick had been rear ended in a motor vehicle accident. He felt the images could have been related to that potential trauma. However on

the balance of probabilities he concluded that the findings were compatible with different things including trauma of any age, past inflammation which could be minor like minor viral meningitis or some other problem like blood pressure increase.

[213] He was asked to assume that there was abnormal blood flow and demonstrable left-handed weakness in Hornick. The question was whether the right orbito-frontal lobe anomaly would cause, on the balance of probabilities, left-handed weakness. He said he could not necessarily draw that conclusion because the affected areas of the brain were not motor control areas.

[214] He was asked to assume Hornick had an abnormal blood flow and memory loss. Could the anomalies cause memory loss? He said it was a "low to very low probability". He did say that when there were lesions in the pre-frontal area one could expect a variety of symptoms. Disinhibition would be one, lack of judgment, incapacity to plan your actions or make organized decision towards what the person would do were others.

[215] Regarding Dr. Reid's opinion that it was 70 to 75 percent probable that the blood flow studies showed a lesion caused by the motor vehicle accident, Dr. Soucy said his assessment of the case did not accord with Dr. Reid's. He could not come up with a number as Dr. Reid. He was not even convinced there was a lesion. The anomalies or irregularities did not convince him that there were definite abnormalities.

[216] Even with the computer analysis that was used, he would call the scan normal. There were irregularities but it was highly unlikely, even knowing that Hornick had been in an motor vehicle accident, that the irregularities were attributable to the accident. The neurological clinical information did not change the nature of the images. The images remained the same.

[217] It was possible the accident could have caused the irregularities but it was not a predominant possibility based on what he could see here in the scan.

[218] He admitted he did not know the details of the rear end accident or how significant the transfer of kinetic energy was. He did consider the fact that Hornick had lost consciousness. That indicated significant head trauma.

[219] It was his opinion that he could not date or age the deficit. It could have been there for 30 years or 2 months. He had no way of differentiating it. The only way to know for sure was to have a prior scan just before the accident and compare the changes if any.

[220] On the totality of the evidence he felt it was possible that there was an abnormality but the degree of what he saw did not convince him that it was clearly abnormal. It could be a normal variant. It could be something that would be picked up in patients who had no complaint whatsoever. There were normal variants that could be quite

pronounced, for instance, if 100 patients were scanned out of a reference population, five percent of those would have exactly the same irregularities. That was the definition of the five from the standard deviation. It was not a large number, but it was still a normal observation.

THE LAW:

THE THRESHOLD APPLICABLE TO GENERAL DAMAGES:

[221] Section 267.5 as amended by Bill 59, governs the substantive elements of tort claims associated with motor vehicle accidents. The section sets out the test that has to be met before an injured victim will be allowed to sue for non-pecuniary loss.

[222] Despite any other act and subject to subsection 6, the owner of an automobile, the occupants of an automobile and any person present at the incident are not liable in an action in Ontario for damages for non pecuniary loss, including damages for non pecuniary loss under clause 61(2) of the Family Law Act, from bodily injury or death arising directly or indirectly from the use or operation of an automobile, unless as a result of the use or operation of the automobile, the injured person has died or has sustained.

- a. permanent serious disfigurement; or
- b. permanent serious impairment of an important physical, mental or psychological function.

THE CASE LAW:

[223] *Meyer v. Bright* [1993] O.J. No. 2446 remains the seminal case in Ontario in dealing with the meaning of the words in the limitation sections of the Insurance Act. The Ontario Court of Appeal held that the limiting words in the exemption should be given their ordinary and natural meaning. The onus is on a plaintiff as to whether he or she has satisfied, on balance, that he or she comes within one of the statutory exemptions. No case since *Meyer* has indicated that the regime in Bill 59 cases introduced any element not before the court in *Meyer*. The court in *Meyer* set out the three well known questions that need to be answered.

1. Has the injured person sustained permanent impairment of a physical, mental or psychological function?
2. If yes, is the function which is permanently impaired an important one?
3. If yes, is the impairment of the important function serious?

[224] In *May v. Casola* [1998] O.J. No. 2475 the Ontario Court of Appeal dealt with a case where an appeal had been made by the plaintiff from a decision of a motions

judge that she was not permanently injured in a motor vehicle accident, and that her injuries did not constitute a serious impairment. The plaintiff experienced a sleep disorder, severe neck pain, headaches, and nausea as a result of the accident. The appeal was allowed. The court found that the plaintiff's symptoms had a significant effect on her enjoyment of life, and constituted a serious impairment. The expert's opinion that the injuries were not permanent was directly opposed to the parties' agreement that the injuries were permanent.

[225] Carthy J.A. found that a person who can carry on daily activities, but is subject to permanent symptoms including, sleep disorder, severe neck pain, headaches, dizziness and nausea which, as found by the motions judge, had a significant effect on her enjoyment of life must be considered as constituting serious impairment. He said:

In some cases, it is impossible to attribute a single cause for injuries and the traditional test for causation, the "but-for" test, is unworkable in some situations. In those cases, of which the immediate case was an example, the traditional test is replaced by the "material contribution" test, which is concerned with whether the defendants' negligence "materially contributed" to the occurrence of the plaintiff's injuries. Unfortunately, the trial judge, in his instructions to the jury about contributory causation, did not refer to: the post-accident condition; a potential cumulative effect from the injuries she sustained in the motor vehicle accident and her pre-accident condition; or the requirement that the defendants be held liable for the plaintiff's full loss if the jury concluded that her overall post-accident condition resulted from such a cumulative effect. The trial judge did not invite the jury to consider and determine the pre-accident condition and compare it to the post-accident condition.

The jury's award of general damages indicated that it found that the accident materially contributed to the appellant's post-accident condition. Having *[page366]* established causation, it was the plaintiff's burden to prove the nature, extent, and duration of the post-accident condition that she claimed resulted from the accident or alternatively, from the combined effect of the accident and her pre-accident condition. It was open to the jury, having found that the accident materially contributed to the appellant's loss, to accept the defence theory and conclude, on the whole of the evidence, that the plaintiff had sustained only a moderately severe injury, the overall effects of which had been resolved within a few years.

In *Athey v. Leonati*, the plaintiff was injured in two successive motor vehicle accidents. Thereafter, while he was recovering from those injuries, he experienced a disc herniation while exercising. The trial judge found that the herniation was caused by the combined effect of the injuries that the plaintiff sustained in the accidents and his pre-existing back problems. She apportioned the plaintiff's loss between the tortious (the accidents) and non-tortious (the pre-existing back problems) causes, and awarded the plaintiff damages of 25 per cent of his full loss.

Although the plaintiff's appeal to the British Columbia Court of Appeal was dismissed, his further appeal to the Supreme Court of Canada was successful. In my view, the judgment of the Supreme Court makes clear that the legal principles concerning contributory causation flow from at least two foundational tort law rules. First, a defendant is liable in tort for all injuries caused or contributed to by his or her negligence. Second, as described by Major J. on behalf of the court (at p. 472 S.C.R., p. 243 D.L.R.): (Emphasis in original) **[page374]** In addition, as relevant to this case, the following causation principles were identified by **the court**:

(ii) however, to ground liability, the defendant's negligent conduct must have "materially contributed" to the occurrence of the plaintiff's injury. As indicated by Major J., at p. 466 S.C.R., p. 239 D.L.R.: "A contributing factor is material if it falls outside the de minimis range";

(iii) separation between injuries and, hence, division of liability among joint tortfeasors or assignment of liability to one tortfeasor for less than 100 per cent of the plaintiff's injuries is permissible where some of the plaintiff's injuries have tortious causes and some do not. That division or assignment, however, is not possible or appropriate where the plaintiff's loss and its consequences, in reality, are one injury;

(iv) a tortfeasor is liable for the plaintiff's injuries even if the injuries are unexpectedly severe owing to a pre-existing condition (the so-called "thin skull" rule); and Major J. also addressed the ramifications, in the *Athey v. Leonati* case, of the principles which he had outlined (at pp. 475-76 S.C.R., pp. 245-46 D.L.R.):

1. If the disc herniation would likely have occurred at the same time, without the injuries sustained in the accident, then causation is not proven.
2. If it was necessary to have both the accidents and the pre-existing back condition for the herniation to occur, then causation is proven, [page 375] since the herniation would not have occurred but for the accidents. Even if the accidents played a minor role, the defendant would be fully liable because the accidents were still a necessary contributing cause.
3. If the accidents alone could have been a sufficient cause, and the pre-existing back condition alone could have been a sufficient cause, then it is unclear which was the cause-in-fact

of the disc herniation. The trial judge must determine, on a balance of probabilities, whether the defendant's negligence materially contributed to the injury.

(Emphasis in original)

Athey v. Leonati confirmed that once it is proven that a defendant's negligence was a cause of the plaintiff's injury, whether demonstrated directly or by inference of a causal connection, a damages award should not be reduced to recognize the contribution of non-tortious causes to the plaintiff's loss. In establishing that principle, the Supreme Court restated, and affirmed, many basic tort law principles and the earlier judgment of that court in Snell v. Farrell, [1990] 2 S.C.R. 311, 72 D.L.R. (4th) 289.

EXPERT WITNESSES:

[226] A comment should be made about expert witnesses. Vern Krishna, former Treasurer of the Law Society of Upper Canada wrote an article commenting on experts in long trials. The following were the pertinent parts.

As commercial trials become more complex, lawyers must increasingly rely on experts to explain scientific, financial and tax matters to trial courts. Most commercial litigation requires expert testimony on business valuation and tax issues. The purpose of expert testimony is to assist those trying cases to make an informed judgment by providing special knowledge that the ordinary person would not know.

In theory, the expert is an impartial witness and not beholden to the party who retains him or her. In fact, some experts are partisan and beholden to their retainers. This minimizes their value as assistants to the court.

Courts generally allow expert testimony based on relevance, necessity and the qualifications on the expert. The cornerstone of such testimony is the independence of the expert. Where the court cannot rely on the impartiality of the expert, the trial becomes a battle between opposing experts, each carrying the banner for their retainers.

An expert is not a hired gun. Regardless of who pays the expert, his or her primary duty is to the court and not to the client. Unfortunately, it is difficult for the expert to eradicate from his or her mind exactly who was doing the feeding. There are no easy solutions to resolving the hired gun problem.

The problem, however, is that it is not easy to define the line where an expert ceases to be an independent person and becomes a partisan witness. In a recent British decision, Mr. Justice Jacob lambasted two professional accountants, one a senior insolvency partner, over a report in a trial. The judge said the accountant had "failed in his duties to the court" and "unconsciously.... espoused his clients cause".

[227] These comments were important as there was a battle of the experts in this case especially between Dr. Sweeney and Dr. Shapiro. I do not find that the mistake made by Dr. Sweeney regarding the date Hornick told him about an insurance settlement, of any negative import in determining his credibility. It just was not important.

[228] I do not accept Dr. Shapiro's opinions as she should not have conducted the testing knowing that Hornick was not amenable to properly entering and completing the testing. Any results obtained would be skewed and, in the absence of more meaningful direct testing for feigning, faking or misleading, her results were not acceptable. Hornick could only come out of that testing session having created a very negative impression on Dr. Shapiro and he employee. He was doomed to a negative report. She became dangerously close to being an out and out advocate for the defence seeking to "dig out some mud" regarding Hornick in order to discredit him, instead of properly applying the tests and evaluating the results.

[229] I accept the evidence of Dr. Sweeney that she should not have conducted the tests. This finding was enforced by the position of Dr. Kuiack who said she would not have done the testing because Hornick was not predisposed to participate willingly in order to try to do his best and get the appropriate results. Dr. Sweeney agreed with Dr. Kuiack.

[230] Dr. Reid and Dr. Soucy are to be complimented in their approach to the Spect scan evidence and its importance. They disagreed but did so on the basis of the same test results. I prefer the evidence of Dr. Reid as the clinical psychiatric and psychological observations and testing supported his decision and served as a check and balance. Dr. Soucy's evidence, standing alone, and without the support of the neurological testing and consideration of the dynamics of the horrible crash, would be acceptable in its own encapsulated form. It can't be accepted when looked at in the totality of all of the evidence of brain injury.

THE ISSUES:

Was Hornick a credible witness?

[231] Mr. Hornick's credibility was central to the determination and extent of his injuries and his ability to work. Most of Mr. Hornick's complaints were subjective. Was he credible in his description of his pain and his limitations?

[232] Mr. Justice McKeown in *Victorov v. Davidson*, [1998] O.J. No. 190 at p. 15, said the “best judicial test of credibility is a consistency and harmony or lack thereof with a preponderance of probabilities disclosed by the facts and the objective evidence.” I accept that statement as being a good judicial expression of how to test the credibility, believability and reliability of a witness’s evidence.

[233] The defence attacked Hornick’s credibility concerning the description and extent of his pain and disability as well as his inability to work. I will answer the allegations on a point by point basis.

[234] First, Hornick misrepresented the nature of his relationship with his wife before the accident. He tried to tell Dr. Sweeney that he had a satisfactory relationship with his wife but they had their problems. He led Dr. Sweeney to believe the relationship with his wife was within normal limits. Hornick may well have believed his relationship was within normal limits because his judgment was clearly not functioning within normal limits when he spoke to Dr. Sweeney. Pam also indicated the relationship was within normal limits notwithstanding the emotional and physical abuse she endured. There was no intentional misleading by either Pam or Hornick. That was the way they each viewed their marriage.

[235] After the accident, Mr. Hornick’s anger and obsession was transferred to the insurance companies, the defendants, their lawyers and, after their separation, to his wife. I agree that Hornick did have these apparent obsessions. However, when a piece by piece analysis is undertaken on evidence given by a person who was obviously angry with the system and all involved in it and was clearly somewhat dysfunctional in his day to day existence, this reaction is understandable and does not necessarily affect his credibility. It is but one factor to consider.

[236] I pause here to comment that one piece of evidence may well be determinative of a person’s credibility but usually the finder of fact needs to determine credibility on the totality of the evidence. The finder can accept all, some or none of a witnesses evidence.

[237] Uniformly, Hornick cooperated with the professionals to whom he was sent by Mr. Allin but intentionally did not cooperate with the professionals appointed by persons adverse in interest to him. I accept that comment as true especially in his interaction with Dr. Shapiro but in all fairness to Hornick he had been put through the mill in terms of attending, answering questions and taking tests. It was quite natural for a man of his mental and physical make up, post accident, to react that way. He cannot be judged in the same fashion as one who was not impaired physically, mentally and psychologically.

[238] Hornick deliberately underperformed on some of the tests administered by Dr. Shapiro. First she did not administer the tests, her assistant who did not testify did the

administration of the tests and Hornick was not asked about his rapport with that person. It was clear he did not want to be there and participate. Both Dr. Sweeney and Dr. Kuiack would not have administered the test to Hornick in conditions where he was not appropriately motivated to complete the testing. This was normal and I make no negative finding against Hornick for whatever he did on the day he was tested at Dr. Shapiro's office.

[239] Hornick misrepresented the extent of his earlier right knee injury. He testified that the knee injury was resolved in months. He told Dr. Yovanovich that his right knee resolved over a period of "*two to three months*" (exhibit 7, tab 20, page 18). But, Dr. Yovanovich knew, because he treated Mr. Hornick, that his right knee symptoms were "*present for a period of more than two years following work-related injuries*". Why would he mislead his former doctor who had seen him at least twice? This was not a deliberate attempt to mislead.

[240] Hornick denied any continuing problem with his right knee but his wife testified that his right knee gave out when he was walking in the mall. There was no date that I recall given by Pam regarding this event. The evidence of his family and his co-workers was important in determining this particular allegation. There was no merit in it in and of itself.

[241] Dr. Pepin examined Mr. Hornick on August 16, 1999 and stated "it is clear that this gentleman on structural testing on physical examination has not sustained any serious injury to his musculoskeletal system. There were some discrepancies in his physical examination, such as his decreased range of motion as compared to the videotape ... there also was the decreased sensation on the whole of the right side of his body which is not in keeping with any particular dermatomal pattern and certainly not in keeping with any known anatomy or physiology". I watched the video carefully and agree wholeheartedly with Dr. Teasell's analysis of what occurred and don't accept Dr. Pepin's opinion on Hornick's activities on the videotape.

[242] Dr. Clifford gave precise and particularized evidence about Mr. Hornick's restriction in motion which he concluded were "*hesitant in the cervical spine and in the thoracic spine.*" Clifford was not cross-examined on his opinion that Mr. Hornick's movements were jerky and not smooth. This evidence is not that inconsistent with the evidence of Dr. Teasell in terms of the range of motion and hesitancy.

[243] Dr. Clifford said that on the videotapes, Mr. Hornick's movements were freer than in his office. I don't know the import of those comments because I was not there to see but a picture is worth a thousand words and I saw what I saw in the video and Hornick did not have free movements like a person without physical limitations or impairments.

[244] Hornick seeks future loss of income on the assumption he intended to work to age 65 even though he told Dr. Sweeney he intended to retire at about age 60. This comment does not reflect the realities of the work lifetime of a labourer like Hornick who had no asset base to look to when he retired. The comment to Dr. Sweeney was unrealistic and not well thought out. Hornick may not have been able to conclude his age of retirement because of the situation he was in during the course of this litigation. Sixty-five is a more realistic age of retirement in the circumstances of this case.

[245] Hornick craves attention and his continuing complaints and bizarre constellation of symptoms have given him attention from Mr. Allin, a host of experts, the insurers, their lawyers and this court. Hornick was not that smart or manipulative to manufacture the bizarre constellation of symptoms.

[246] Dr. Sweeney explained that Mr. Hornick's anger and obsession towards the insurance company would, in his mind, justify embellishment and exaggeration. He did not say he believed Hornick was embellishing or exaggerating that I recall.

[247] Hornick experienced an intensity of symptoms that were abnormal and unusual. I don't think any witness can be exact in his or her regurgitation of facts over time. If there were not some discrepancy either in intensity or forgetfulness I would be surprised and would not accord credibility if the facts were identical each time they were recited.

[248] Dr. Sweeney said an untrue statement by Mr. Hornick about the extent of his knee injury would be consistent with his anger and obsession towards the insurance company. This does not surprise me. I think Hornick had had it mentally as this case slowly moved its way through the system. No negative inference flows from this comment. Dr. Sweeney did not draw any adverse inference against Hornick and neither do I.

[249] Dr. Sweeney said that "there are several non-medical factors which are playing a more important role in Mr. Hornick's decision to disqualify himself from the workforce." These factors are anger, obsession, lack of motivation and a belief in entitlement. These would be part and parcel of his psychological reaction and not his physical difficulties. These reactions would be part of most disadvantaged person's reactions to work if they were physically, mentally or psychologically impaired as I have found Hornick to be after the crash.

[250] Anger, obsession, motivation and gain through litigation are all factors relating to recovery. I don't disagree but these factors plus all the usual considerations must be analyzed to determine credibility.

[251] Finally, I must also consider that I had the opportunity of observing Hornick during the trial, coming in and out of court and walking from the body of the courtroom to the witness stand. He continuously walked with a pronounced limp favouring

his right leg. He had had to sit and stand while testifying and the time was split about 50% sitting and 50% standing. While standing he would turn to face me while answering a question and had difficulty in turning his head. He had to ask to leave the courtroom several times when he was testifying in order to go to the washroom. Again I could see him walk and move and all movements were consistent over the extended time of this trial.

[252] He had difficulty in following some of the questions asked of him in cross-examination. He would get frustrated and aggravated with defence counsel when counsel appeared to be asking the same question over and over again. His body language indicated he was getting upset. He was unable to control himself and his reactions were consistent throughout his lengthy examinations.

[253] I have considered the evidence as a whole including the evidence of his family and coworkers who all testified that before the crash he was a decent, hardworking, non complaining honest fellow who had some interpersonal problems with his wife. No one said he exaggerated, lied or embellished. Post-accident all said he had some serious problems mentally and physically that made him a changed man. I cannot lose sight of this evidence and must factor it in when weighing the cumulative effect of the evidence and the allegations concerning his credibility.

[254] Finally I have considered the evidence of Dr. Teasell and Dr. Sweeney concerning the question of Hornick tricking or deceiving them. I accept their opinions when they said it was possible but I conclude they thought it not probable. Of particular import was Dr. Sweeney's comment that Hornick took the neuropsychological tests 2 years apart and the results were consistent, one test to the other, and that could not be faked.

[255] I have reflected on the test as set out by McKeown J. concerning credibility. I find a consistency and harmony in Hornick's evidence at trial and in his reports of his difficulties both psychologically and physically. I am satisfied after considering all the evidence and the reports of the various professionals that Hornick was and is a credible person and not prone to deliberately exaggerate or embellish his injuries or his situation.

Hornick's physical impairment if any and the matter of contributory causation.

[256] After reviewing the doctor's evidence and a number of their reports and comparing the evidence and reports I find that I prefer and accept Dr. Teasell's evidence without reservation. He was a foremost authority in physical medicine with expertise in neck, back and knee pain. His observations, analysis and opinions were in accord with the preponderance of acceptable evidence introduced from Dr. Patel, Dr. Allen, Ms Bertoia and Hornick's coworkers. He gave very careful thought to his opinion. He was vigouresly cross-examined and was not negatively affected by any questions put to him. He was forthright and his evidence was credible, believable and reliable.

[257] The evidence of Hornick's family, friends and coworkers also was helpful in assisting the determination of his "before and after" physical make up and abilities.

[258] I reject Dr. Clifford's observations and opinions because he seemed to premise his findings on the basis that Hornick could have been misleading or amplifying his pain for improper or compensatory gain. I find that attitude skewed his findings and opinions. I have found Hornick to be credible in his verbalization of the non-objective pains he was suffering.

[259] The cumulative affect of all of the evidence allows me to find on the balance of probabilities that Hornick suffered a permanent serious impairment of the following important physical functions namely, the use of his neck and lower back. The impairment was exacerbated by the knee injury that caused him to limp 5 years later. The impairments were significantly serious.

[260] The issue of contributory causation was raised because of his previous history of knee and back injuries. It was possible that the injuries could have materially contributed to the complaints post accident. I find that the previous injuries were remote in time and were not a significant contributory cause. I don't accept Dr. Yovanovich's opinion as to the present rationale for Hornick's pain in the knee. There was no evidence from a back specialist concerning Hornick's back. Dr. Teasell did not agree that some form of orthopaedic intervention was required.

[261] Additionally Dr. Patel did not record any complaints from Hornick on his knee since 1985 and his back since 1992. In particular the back difficulties had completely resolved themselves in 1992. I do not find that the present injuries were unexpectedly severe owing to pre existing conditions in the knee or back. Further, I find that his knee injury in 2001 was not and should not have been factored into any findings or opinions between the accident in December of 1997 and the time he fell at his mother's house.

[262] The question of whether the current complaints were related to the crash was easily determined. The evidence of his family, friends and coworkers was extremely helpful as it established a benchmark of previous abilities. All concluded his present physical complaints were post accident. This evidence was bolstered by the comments of Drs. Allen, Teasell, Hoffman and Sweeney who all felt the complaints were attributable to the crash.

Was there a permanent serious impairment of an important mental or psychological function?

[263] The Plaintiff led evidence from a psychiatrist and 2 psychologists. Dr. Hoffman testified that, after reviewing medical, physiotherapy, psychological and neuropsychological reports as well as the pictures of the crash, interviewing Hornick and

considering the views of Pam, he was of the opinion, on the balance of probabilities, that Hornick suffered a brain injury to the right frontal lobe of his brain in the crash. He was supported in his views by the testing of Drs. Archibald, Casey, Sweeney, Leenaars, Smith and Reid.

[264] He was satisfied that Hornick had sustained a **permanent mental impairment** because he had a loss of executive functioning which concerned his ability to do multi-tasking, to plan ahead and to do complex thinking. He had lost the ability to maintain his self-esteem which most people take for granted. As well, he had lost the ability to maintain his mood in a calm and relaxed manner.

[265] Regarding whether Whitney Hornick had suffered a **permanent impairment of an important psychological function** as distinct from a mental function the doctor indicated that “I’m afraid I cannot distinguish mental from psychological. I recognize the law includes both, but as a psychiatrist I don’t know the difference between mental function and psychological function”.

[266] He agreed that the above opinion would cover both the mental and psychological functions.

[267] Regarding whether there would be any change in the foreseeable future in these impairments and whether the plaintiff would improve in the foreseeable future, the doctor opined that “they are unlikely to improve in the foreseeable future. The reason given was because “in emotional or psychiatric states, the longer you get caught up in emotional suffering, then the longer it continues. We don’t know whether that is because a person develops the habit of being there, of just suffering, being anxious, depressed and angry or whether it’s because the environment changes and you lose supports the longer you stay there or whether in fact such states even produce a change in the transmitters of the brain that make it difficult to overcome”...” Our moods, every thought must have some chemical change in the brain for which we have absolutely no explanation, but the longer a person gets caught up in psychopathology the more likely it is to continue”.

[268] He was not asked to give a direct opinion on the question of seriousness and the importance of the impairment but I can conclude from totality of his evidence that he was of the view that the impairments were serious and to an important mental or psychological function.

Was there a brain injury?

[269] The difficulty with a brain injury was that a person can’t feel it, taste it or smell it. It might be able to be seen on a Spect scan. In order to determine whether there

has been a brain injury, the determiner must take into consideration all of the evidence both subjective and objective in order to make the finding on the balance of probabilities. Hornick's pre accident status was that he did not have a brain injury although he may have had some limited cognitive difficulties. However, he was able to function at Duo-Matic and supervise people, put up with whiners and exhibit interpersonal communication skills and be responsible. He would not lose his temper and seemed capable of completing his chores satisfactorily. He was a hard worker and seemed to live a normal family life notwithstanding leveling some emotional abuse on his wife.

[270] Drs. Reid, Sweeney and Hoffman were satisfied on the balance of probabilities that Hornick suffered a brain injury in the crash. I did not accept any evidence to the contrary. Dr. Soucy's evidence was one-dimensional and did not take into account at a high enough level the neuropsychological and psychiatric evidence, or the effect of the dynamic force of the kinetic energy when he gave his opinion that he could not conclude that Hornick had suffered a brain injury in the crash. I reject his evidence on this point.

[271] Dr. Archibald felt it was probable Hornick suffered a mild traumatic brain injury in the crash.

[272] Dr. Sweeney was of the view that Hornick did suffer a brain injury and was moderately impaired in regard to certain functions especially in regard to problem solving. He felt Hornick could not have had those impairments when he was gainfully employed because he would need those skills to do his old job at Duo.Matic.

[273] He opined that Hornick had suffered a mental impairment that included cognitive impairment, emotional dysfunction and psychological difficulties. Hornick was significantly and seriously impaired mentally.

[274] He was asked if he had an opinion as to whether the plaintiff had suffered a psychological impairment. It was his view that a psychological impairment was included in a mental impairment because the words mental impairment were more general. The words "psychological impairment" included "neuropsychological impairment", because neuropsychology comes from psychology. It was his opinion that the plaintiff did have a psychological impairment because he was moderately impaired neuropsychologically. Also, he has an emotional control problem that also was a psychological impairment. In the end it was his opinion that the plaintiff had a significant and serious psychological impairment.

[275] Regarding the question as to whether the mental and psychological impairments were important to the day-to-day function of Hornick, it was his view that these cognitive difficulties were going to "seriously diminish the quality of this man's life" and so were important and serious.

[276] He concluded that these mental and psychological impairments were to important bodily functions and were permanent. Further, he determined Hornick was

suffering the ongoing debilitating effects of significantly diminished neuropsychological status as a consequence of head injury in the motor vehicle accident.

[277] I reject the validity of Dr. Shapiro's evidence because Hornick had not been properly prepared by her and/or the psychometrist before he was tested.

[278] Dr. Reid felt he had suffered a brain injury and the Spect scan was very helpful in determining brain injury from an objective point of view.

[279] After considering all of the evidence and opinions I conclude on the balance of probabilities that Hornick suffered a permanent serious impairment of an important mental and psychological function. Drs. Reid, Hoffman and Sweeney felt these injuries were all attributable to the crash.

[280] I find that the degree of the seriousness of the impairment was moderate.

[281] I also find that in law there is a difference in the meaning of mental or psychological function notwithstanding the experts felt they may well be subsumed one in the other. Parliament meant there to be a difference and mental may well mean the aspect of actual brain injury and psychological meant the psychiatric or psychological aspect. I am satisfied on the totality of the evidence that the opinions of the experts sustained my findings concerning the question of whether Hornick suffered a permanent serious impairment of an important mental or psychological function.

Was there a failure to mitigate?

[282] I am satisfied Hornick made serious efforts to rehabilitate himself physically through physiotherapy and a diligent following of Dr. Patel's recommendations.

[283] He attended the appropriate government job search offices and gave reasonable explanations for not going on a daily basis. I accept his reasoning in not going every working day. He was somewhat unreasonable in his responses to questions about work during the trial. He had a valid point when he asked why he had not been retrained. I think he realized he could not do the jobs he did before the crash and needed to be vocationally upgraded if he could.

[284] I think he was motivated to go back to work and the end of this litigation may well afford him the chance to get some vocational retraining and obtain a job within his physical and mental capabilities.

[285] I do not accept the defence position that Hornick had no motivation to go back to work because he was making more money not working than he did when he was working. He could not now work at a job commanding the same physical and/or mental abilities as the jobs he had before the crash. He just could not do the same old jobs that he

did at Duo-Matic or Millers. The expert evidence was clear that he could only do relatively simple jobs that did not demand a higher level of physical or mental dexterity.

Was Hornick a malingerer?

[286] Dr. Sweeney was asked if Hornick was a malingerer. He responded that “the client’s cooperative, highly motivated test behaviour, the meaningful clinical profile demonstrated, and the absence of positive psychometric indices of malingering rule out any possibility of exaggeration or outright malingering.”

[287] I reject Dr. Shapiro’s evidence that Hornick was feigning and Dr. Clifford’s evidence that he was being less than truthful.

FAMILY LAW ACT CLAIMS:

[288] A thorough review of Pam’s evidence respecting this claim was necessary notwithstanding that it may appear to be repetitious.

[289] 38-year-old Pam testified that she completed her high school education at age 17 while in grade 12. She married Hornick on October 6, 1984 and they spent the majority of their married life in Tilbury, Ontario living at 10 Park Lane where she still lives. Following the marriage, Pam worked part time outside of the home and since 1989 has been working as a health care aid on a part-time basis. Pam and Hornick planned to have a family and Donald was born in 1989 and Dylan in 1994.

[290] She described her life with her family as a normal marriage. She did the housework, cooked, maintained the home and looked after and raising the kids as well as doing the banking and paying the bills. Hornick helped with the cooking, cleaning, laundry, yard work, snow removal, grass cutting, outside cleaning of the windows and doing repairs around the house.

[291] The family [including the children] would play, swim, have campfires, ski doo, skate, toboggan, bike, camp and ice fish together. They attended car shows in the summertime. They spent a lot of time socializing with his family mostly and frequently took holidays camping in Wheatley, Ontario. They organized celebrations for the children and took care of their children’s physical and other needs.

[292] As a couple they were jointly engaged in the management of their financial affairs and made joint decisions about purchases. For example, Hornick wanted to purchase a classic car, and Pam co-operated by assisting in obtaining a loan for that purpose.

[293] Hornick was a strong man who was very capable at doing big and little chores. She was aware that he had hurt his right knee playing hockey as a boy before they got together in 1983.

[294] He never took any time off work because of his right knee during the five years before the accident. He never complained of pain in his right knee but she could see him rubbing it on the odd occasion and he might take Tylenol. He never walked with a limp at any time in the five years before the crash. There were no complaints of back pain either.

[295] When he worked in Wallaceburg at Duo-Matic, he would have some bad days but when he began working at Miller just before the crash, he enjoyed that job and was happy.

[296] Pam also reflected the reality of their relationship and indicated they had arguments and disagreements during the marriage but these would be resolved and they would move on. She considered her marriage before the crash a normal one. However, she agreed that she had consulted a lawyer before the accident because of occasional matrimonial difficulties but decided to stick it out notwithstanding the difficulties.

[297] She agreed that for about 10 years before the accident, Hornick would swear at her, call her names, argue heatedly with her, put her down, belittle her, make fun of her and showed little or no empathy towards her whatsoever. For instance, if she wanted to read in the evening, he would not understand that desire. He was jealous that she wasn't communicating with him. She agreed he was quick to anger before the accident.

[298] He was also angry at her family. He repeatedly complained about her mother. He would get upset when his children would spend time with her family. He would erase telephone calls that had been left for Pam on the phone from her family.

[299] To Pam and Hornick this was a relatively normal marriage. It may not have been for many observers, but it was for them.

Post accident:

[300] After the accident, she first saw Hornick at about 7 p.m. in the hospital and sometime later they left together. Once at home he took drugs for pain and mainly sat around or lay on the couch. He would complain about his knees, shoulder and awful headaches. She could tell by his facial gestures that he was in pain.

[301] Duties changed around the house and she would do the snow removal and cut the grass. He could load the washing machine and do the laundry. He could no longer look after the pool. Sometimes he would assist in cutting the lawns.

[302] She noted changes in his behaviour. He did not like dealing with doctors and filling in forms. It frustrated him. He was depressed and sad, slept more than usual and was bitter because of the accident as it had disrupted their lives. She continued to work a bit.

[303] Hornick began to do some bizarre things. For instance, twice he undid the battery in her car so she could not get it started. She confronted him and he said it was his car and he could do what he wanted because it was in his name.

[304] His anger had a lot to do with their separation. He would get mad and stay mad. The kids would set him off especially if they came in the house with dirty shoes. Before the accident that would not have bothered him. Little things would bother him and he would curse and swear but would not in front of the kids.

[305] Before their separation in 1999 Pam went away on a vacation with three of her girlfriends from high school. She had told Hornick about the trip a few months before she went and he was not happy. He told her that he hoped the plane would blow up with all of them in it. They were gone eight days and when she got back Hornick was not too receptive towards her. She was sitting in a chair downstairs and he came up behind her, tipped the chair over and she went flying. The next day he took a cigarette and burned her arm. He hid her souvenirs that she had brought back and broke some of them. The kids were not present during these altercations. He accused her of having an affair. One day she took a dust mop and hit him. On October 27, 1999 they separated and there has been and will not be any reconciliation.

[306] After separation she lived with her parents. Then she went back to the matrimonial home with the kids and stayed there. At the date of trial Donald was living with his father, and Dylan was living with his mother. Donald left because of an argument with Pamela about a laser and has not lived with her since. Dylan has some health problems.

[307] Donald does not speak to Pamela, doesn't confide in her but does come over to her house for short visits. There is no discussion between Hornick and Pamela regarding parties, holidays, gifts or about school activities. Occasionally they do write notes back and forth about when the kids will visit.

[308] Hornick sold his one half interest in the matrimonial home to Pam's boyfriend who is now cohabiting with Pam and has done so since November 2002. They have a permanent relationship.

Pam's position:

[309] As of December of 1997, Pam described her husband as genuinely happy with his work with Millers' on the highway and, despite any doubts that she had about the marriage before that date, she was relatively satisfied with the marital relationship, was deriving the benefits obtained from that relationship in her life and had not decided to leave notwithstanding the mainly emotional abuse that occasionally was directed at her.

[310] Following the accident Pam described her husband as being sad, depressed, frustrated, easily angered, jealous and someone who acted out to the point of significant physical violence towards her.

[311] This growing fear of her husband over time was consistent with someone living with a person who was socially disinhibited. Dr. Hoffman talked about persons with a frontal lobe injury that, among other things, have problems with executive functioning and, may also become socially disinhibited. Whatever restraints were in place before the accident, in terms of his being able to control his behaviour towards Pam, were removed and she became the object of his acting out.

[312] Even though they continued to share a common bedroom following the accident up to the time of separation, ultimately, the strain of Hornick's changed behaviour destroyed their relationship.

Pam's loss:

[313] Among other things, she lost the ability to communicate in any meaningful way with her husband about matters important to her and the children. She lost Hornick's care, companionship and guidance with respect to the usual pre accident manner of dealing with all kinds of matrimonial matters that were fundamentally important to her. For instance there were no more discussions about family matters, financial matters, social and recreational matters, interpersonal relationship matters and children's matters, all of which were the cornerstone of the normal typical married life this couple had before the accident and which was now irretrievably lost.

[314] It was Pamela's position that, as a result of this accident, she lost her husband and her marriage and for those losses she deserves compensation for loss of care, companionship and guidance.

[315] The totality of the evidence reflected that Pam and Hornick lived a relatively stable family life. They had the benefit of mutual care, companionship and guidance with respect to all of those matters involved in a typical simple marriage like they enjoyed notwithstanding some marital difficulties.

[316] Counsel submitted that Pam's loss was a substantial one, having regard to the lengthy marriage and the on-going permanency of the loss and the award should be in the range of 40,000.00 to \$50,000.00.

Defence position on Pam:

[317] The Defence took the position that there was a big difference in the evidence in examination in-chief of Pam and the evidence given by her on her cross-examination. The cross-examination was short but was fundamental to understanding both the nature of Pam's claim and the nature of Hornick's claim.

[318] Before the accident Hornick was an emotionally abusive husband. Pam lived with him for less than two years after the accident. Then they separated because of Hornick's unfounded allegations of infidelity and his abusive behaviour. The separation was unrelated to the accident.

[319] There was no claim for services because he was mostly self sufficient in terms of issues of daily living. There was no claim for future care costs on her part or anything like that. She didn't suffer any pecuniary loss. All that was left was a claim for the loss of care, companionship and guidance.

[320] There was no evidence that, during the 22 months of co-habitation after the marriage, she assumed more household or child-care responsibility. In fact the inference was that the contrary was true; that Hornick was there and was having more of an involvement with the children. There was no evidence that their sex life or social life was curtailed. Pam did not have much care, companionship or guidance from this man.

[321] They were two people who were trying to make it because they had two kids. She was trying but he didn't empathize or understand the situation and Pam should not be compensated for "combat pay". This was not a situation where Hornick was totally supportive of his wife, who had a collateral career and he was doing all the things that he could to further her career. There was not any emotional support whatsoever.

[322] Counsel suggested that this case had virtually no value or if any value it would be in the range of \$1,500.00 or \$2,000.00. It was a relationship that at its core had precious little companionship, no guidance and care was non-existent.

The F.L.A. claim of Donald and Dylan Hornick:

[323] At the time of the crash Donald was an eight-year-old little boy who was 13 when the trial started. Dylan was 3 at the time of the crash and was 9 during the trial. No pecuniary loss was claimed for them. They did not render any services. Their claim was for loss of care, guidance, and companionship.

[324] Counsel urged that the situation pre accident was as set out above and the post accident situation was the following, namely; that Hornick was at home in pain running around dealing with doctors. The family money supply had dried up. The house was thrown into chaos. Hornick was not feeling well. The standard of living and the family setting that the children were accustomed to evaporated overnight.

[325] The worst is yet to come for these two boys. They do have a father and his name is Hornick. George, now living with Pam, is not their father. The father that they had at the time of this crash did all kinds of activities with them and was, by all the information available, a good and caring father who gave care, companionship and guidance to his two sons. He is now not that same person and they have lost that care, companionship and guidance.

[326] According to Dr. Sweeney and Dr. Hoffman, Hornick is now prone to exhibiting explosive anger with minor irritants. These two boys are teenagers and common sense tells anyone who has ever been a parent that teenagers can be described as minor irritants from time to time. If they are, then this will affect the boy's relationship with their father particularly if he responds with explosive anger.

[327] These two innocent victims of this crash have suffered about the worst emotional trauma that a child can suffer in that their family unit has evaporated before their very eyes and the breakdown was caused by the crash. Donald doesn't get to live with his mom anymore. Dylan doesn't get to live with his dad anymore. They visit the other parent from time to time.

[328] Therefore the care, companionship and guidance that have been lost can be compensated for by an award to each boy in the amount of \$100,000.00 for their Family Law Act claims.

The defence:

[329] The defence took the position that the children should not be compensated for the break up of their parent's marriage. In effect the children may be better off in the present situation than they were in the old relationship with their father. Hornick is spending more time with Donald now than before the crash and so there is no real loss. Because Dylan is with his mom he is not spending as much time with Hornick but that is his own choice. The claim is not worth more than \$1500.00.

The law:

[330] Most of the cases alluded to dealt in one way or another with the issue of loss of care, companionship and guidance. Plaintiff's counsel conceded that there was no claim being advanced by Pam with respect to compensation for attendant care, nursing or other services provided by her to her husband.

[331] In *Peddle (Litigation Guardian of) v. Ontario (Minister of Transportation)*, [1997] O.J. No. 1874 the court dealt with Darren Peddle [Peddle], a 25-year-old firefighter who sustained devastating neurological injuries in a car crash on a provincial highway in November, 1991. Since the accident he had significant memory loss, drastic personality change and left-sided weakness.

[332] Independent evidence of his pre accident persona was given by Thomas Hackett, age 55, a firefighter with the City of York Fire Department who remembered Peddle as a rookie in very good physical shape. He used to work out in the gym regularly, was easy to talk to and enjoyed playing golf. After the accident, when Peddle attempted to return to work, Hackett noted that he was not the same person. He was very irritable, commonly disagreed with his co-workers and was unable to do the job.

[333] Because Peddle had little or no memory of his wife prior to their marriage, there was considerable friction in the home. He sought solitude, was unable to control his temper and would resort to punching walls. He had very little emotional or physical tolerance and could not cope with the disapproval of others. His attention span was short and he believed he had lost all of his former skills, his patience and his ability.

[334] Annette, his wife, was 27 years old and they had been married approximately six months at the time of the accident. There were no children. After the accident in the following July, Mrs. Peddle had a child that had not been planned for with her husband. She had been contemplating separation because she couldn't cope with her husband and the condition she found herself in. There was a specific finding of fact by the court that separation was inevitable and ultimately the relationship was going to fail. Annette had to take over the management of all the financial matters.

[335] The judge assessed her F.L.A. claim at \$80,000.00. This young woman's life "has been collaterally destroyed by the injuries to Darren Peddle. The services that she has provided to him since this accident, her efforts to rehabilitate him, her countless visits to the hospitals to assist him in every possible way has drawn admiration from all of his caregivers. She has had to endure his temper tantrums, uncontrollable rages, and a loss of all their mutual friends". The judge did not deal with the question of care, companionship and guidance and did not distinguish services rendered from care, companionship and guidance.

[336] There was little question that Annette suffered a loss of care, companionship and guidance. It would have been helpful if that head of damage under the F.L.A. had been specifically identified and analyzed.

[337] However, it appears that the judge in Peddle did not diminish or reduce the award because a separation was being contemplated.

[338] *Ortega v. 1005640 Ontario Inc. (c.o.b. Calypso Hut 3)*, [2002] O.J. No. 1196 concerned a case where a 22 year old young male died as a result of a gunshot wound outside a club in Toronto in July of 1997. The Plaintiff spouse sought \$80,000.00 in damages under the Family Law Act on the following basis, namely:

1. the shocking and brutal nature of her loss combined with her subsequent miscarriage;
2. she would be unlikely to find love again; and
3. she had lost her opportunity to come to Canada and enjoy the benefits of Canadian residency and citizenship.

[339] The trial judge found that those reasons did not necessarily persuade him of such a high entitlement. Their relationship with each other had been relatively

short. Although she testified about plans of marriage, he found that while in Canada, between March and the end of July 1997, the deceased made no efforts to go back to see her, although he found time and money to go and spend a month-and-a-half, more or less, with his brother in New York. There was no evidence that he ever wrote her letters while he was in Canada, although she had forwarded him some five or six letters which manifested her great love and affection for him. She claimed that he phoned her regularly, yet no evidence of long distance charges was introduced to substantiate such telephone conversations. He assessed the value of her loss of guidance, care, and companionship at \$40,000.00.

[340] *Coulombe (Litigation Guardian of) v. Beard*, [1995] O.J. No. 893 was an action for damages for personal injuries arising out of a motor vehicle accident. The plaintiff sustained a closed head injury and a minor organic brain injury, resulting in post-concussion syndrome. He also suffered post-traumatic depression and stress disorder. He experienced deficiencies related to brain functional capacity and personality resources coupled with neck pain and migraine headaches. The evidence established that the symptoms of cognitive and memory deficits displayed by the plaintiff had become so entrenched, seven years post-accident, that total recovery would be grossly delayed if not impossible. It was unlikely he would become competitively employable in his lifetime. At the time of the accident, the plaintiff was employed as a welder.

[341] Prior to the accident he used to do things with his kids (skating, skidooing and fishing). He claimed to have been a "good guy" playing a lot with the children. Since the accident his mood (described by him as "the way you feel") was different. He was no longer alert around the kids were "too fast for me". He described his thinking as "not good" and that he got all mixed up and saw "allusions". He felt very depressed and "pretty scared".

[342] The judge stated at para 63, "there is no question but that the injuries sustained by Michael Coulombe put enormous pressures upon Eleonora Coulombe, leading to a break down of her physical and mental health and resulting in a real loss of guidance, care and companionship from her husband. Because of his injuries, Michael Coulombe declined from the condition of a relatively gregarious, friendly person to a person who was withdrawn, private and depressed, requiring much additional care and attention....for the loss of companionship, care and guidance of her husband, I award Eleonora Coulombe damages in the amount of \$35,000.

[343] The plaintiff in *Bailey v Rycroft*, [1993] O.J. No. 497 was 50 when he was injured in an accident in July of 1987. His spouse was a family physician and they had been together for 15 years at that time. Unfortunately Mr. Bailey's injuries were severe enough and the impact significant enough that he eventually needed permanent institutional care.

[344] The evidence reflected that during the marriage he had provided a substantial degree of support to his wife by way of managing different matters on the home front while she was engaged in her very active full-time physician's practice. Dr Clapperton was awarded \$75,000.00 with respect to her Family Law Act claim for the loss of care, companionship and guidance in this particular case.

Analysis:

[345] The words "care, companionship and guidance" are to be given their ordinary natural meaning. Where the claim is for "care", it must be considered in its broadest sense. It means the providing for the day-to-day needs of another; the providing of some or all of the necessities of life as well as the providing of love, affection and the amenities of life concerning each of the claimants. The key is the providing of the care either voluntarily or if required.

[346] "Companionship" is to be determined by the application of common sense. Here I am speaking of companionship in its ordinary sense, namely, the friendship and the society that a wife or son enjoy with a husband or father. There must be evidence that the relationship was so close that any one of the claimants will suffer because of the loss of that companionship. In other words the claimants and Hornick were usually together at some time during most days and the claimants will suffer without the same exposure to the companionship that existed before the crash and the change in Hornick. This was not a case where the claimants only saw Hornick once in a while before the crash.

[347] Where a claim is by a wife or child for "guidance", it essentially refers to the kind of advice, assistance and benefits a wife or child would receive from a husband or father to assist them in dealing with their everyday activities. This could range from financial, social or sporting advice to husbandly or fatherly advice.

[348] The plaintiffs must prove on a balance of probabilities that they have in fact suffered a loss in being deprived of the elements of care, companionship and guidance they would normally have received from Hornick had he not been injured.

[349] I do not accept the defence theory that the accident did the children a favour by injuring dad. In other words, the fact that another man is living with their mother does not absolve the defendants from compensating the children for the loss of care, companionship and guidance of their dad.

[350] Just because Pam separated from Hornick within two years following the accident is not a matter that ought to result in a reduction of an award to her but rather be considered as a natural outcome that simply reflects the permanency of the loss to Pam in this particular case. Peddle, [supra], recognized that reality.

[351] So often in family law claims the court is dealing with the death of a spouse or father and a "once and for all" award for loss of care, companionship and guidance. A

separation can be as permanent as a death and indeed has other complications associated with it, particularly where there's acrimony and children involved. Here it appears that we now have a dysfunctional situation and Pam and the children's loss is great because of that.

[352] Peddle indicated a relatively high award but care, companionship and guidance was not hived out and dealt with separately. That case is not too helpful in considering an award to Pam.

[353] Ortega was not in as serious a situation as Pam and yet received \$40,000.00 for care, companionship and guidance. Unfortunately the trial judge did not include a thorough analysis of why that amount was chosen.

[354] In Columbe, the court dealt with a long-term relationship where there were two children and the husband-father was completely changed as a result of the accident. An award of \$35,000.00 was made in 1995, some 10 years ago.

[355] Dr. Clapperton received \$75,000.00 because the injured husband was very involved in her career advancement and so was giving her care, companionship and guidance.

Conclusion:

[356] Before the crash the relationship that Pam had with her husband, that I view as a relatively stable but sometimes stressed permanent relationship with some emotional abuse, resulted in a setting where there were two children and a full family life with all of the financial and other family type benefits. I am satisfied on the balance of probabilities that Pam is deserving of an appropriate award for the loss of care, companionship and guidance in the amount of \$45,000.00 less the deductible of \$7500.00 for a net award of \$37,500.00.

[357] Further, I am satisfied the boys have a similar loss of care, companionship and guidance. Before the crash they had an ordinary type of relationship with their father and did all the things an ordinarily attendant father would do living with two boys in town and where they also spent a lot of time on his parents' farm. Their loss is significant and I accept the submission that the worst is yet to come. As a result I award each of the boys \$45,000.00 less the deductible of \$7500.00 for a net of \$37,500.00 each.

DAMAGES:

Quantification of damages:

Diane Blasiak:

[358] She is the director of human relations at the Miller group. She has worked with that group for 15 years. Whitney Hornick was an employee of Miller maintenance. He was initially hired December 9, 1996.

[359] He worked at the Chatham patrol yard as an operator responsible for any jobs concerning Highway 401 like correcting signs, collecting debris, patching pot holes, sanding and snowplowing. "Overtime" was available to him as an operator. It usually was in the winter rather than the summer.

[360] He earned \$14.60 per hour for a 40-hour week. He received overtime when he had worked 55 hours at a rate of 1.5 times \$14.60. In addition, he had a benefits plan for which he paid 50% of the premiums. The benefits covered medical, dental, life insurance, accidental death and dismemberment and long-term disability. He received no bonuses. There was a company pension plan but an employee had to go through a waiting period of two years. The employee would pay 5% of earnings and the company would match the amount. Hornick was not eligible because he did not work for Miller for two years. There was no mandatory retirement age.

[361] After the accident Hornick did not return to work. On May 29, 1998 he was laid off because Miller's Highway maintenance contract was cut back in the spring of 1998 as Highway maintenance was downloaded to the municipalities. He was employed at Miller for just under 1.5 years. There was no union of Miller's.

[362] When on long-term disability, all benefits continue for two years provided the employee pays a portion. Long-term disability continues until worker is back to work or reaches age 65.

[363] Miller lost its contract with the Ontario government in 2002 when its bid was not accepted. A company called T. W. D. took over the contract and asked Miller for permission to contact employees of Miller's to work on the highways as they had the experience. She was aware that some of the salaried workers of Miller were offered jobs by T. W. D.

[364] She was able to forecast what an employee working for Miller after December 17, 1997 would have earned. She looked at a typical operators T-4t form for the years 1998 -- 2001. In 1998 the employee would have earned \$33,028.83, in 1999, \$36,441.53, in 2000, \$36,001.51 and in 2001 \$37,915 43. The contract with the previous government ended March 30, 2002. The benefits did not change and the pension plan remained the same.

What are Whitney Hornick's general damages, if any?

[365] The Plaintiff sought general damages for pain, suffering and loss of amenities of life of \$185,000.00 less the \$15,000.00 deductible or a net of \$170,000.00.

The maximum available as at this trial for a claim for general damages was \$288,700.00 as at December of 2002 according to exhibit 51.

[366] Defence counsel submitted that Hornick had not suffered any cognitive deficit and therefore there was no reason for a significant general damage award. He suggested an award of \$40,000.00. However, he did concede that if there was a finding of a mild brain injury that the damages would be “ratcheted up”.

[367] Counsel submitted a number of cases to support his position that Hornick should not receive any more than \$70,000.00 or so in general damages.

[368] In *Anani v. Gauvreau*, [1995] O. J. No. 1364, the Plaintiff was a 53-year-old airline pilot who was injured in a motor vehicle accident. He suffered a closed head injury with severe depression. He had not been able to return to work because his skills were compromised. The trial judge found he lost his profession and was reduced to living on welfare. At trial he still suffered headaches, pain and other effects.

[369] The trial judge found that it seemed “probable that, apart from his memory difficulties and his difficulty with attention and concentration, his other physical difficulties would probably not substantially interfere with his enjoyment of a reasonable quality of life were it not for the severe emotional overlay and depression resulting from his injuries and job loss. Moreover, since he has been living in Florida with the second Mrs. Anani and, as she did not testify, the evidence as to the impact of his injuries on his recent quality of life is not as satisfactory as it could be, consisting of his own evidence, that of Mr. and Mrs. Leather and the conclusions which may reasonably be drawn from the medical evidence. While the doctors were not hopeful in their prognoses in view of the persistence of his symptoms, it seems likely that the termination of this lawsuit may have some positive effect.” He awarded the plaintiff \$70,000.00.

[370] *Barrett v. Mathews* [1994] O. J. No. 3919 dealt with a plaintiff who suffered a closed head injury and low back pain. He was in a coma for a couple of weeks and suffered mild cognitive impairment and made a remarkable recovery. He was in the low range of intelligence and had worked before but not since the accident. He was awarded \$65,000.00 in general damages.

[371] The plaintiff in *Hest v. Currier* [1995] O. J. No. 175 was injured in sequential car accidents. In the second accident she suffered head, neck and back pain. Memory function was low and there was some indication of brain dysfunction. She was awarded \$10,000.00 general damages in the first accident and \$50,000.00 in the second.

[372] In another case, Mr. Murphy went to a pub and was beaten up. He was found outside and was having trouble breathing. He had a fractured skull and broken facial bones that required extensive surgery. He was left with severe headaches as well as vertigo and intermittent imbalance. He was unable to return to work. He was awarded \$75,000.00 in general damages. [see *Murphy v. Little Memphis Cabaret Inc.*, [1996] O. J. No. 4600]

[373] In *Remouche v. Mercer*, [1992] O. J. No. 900, two fishing boats were in an accident on the Trent River near Trenton Ontario. The plaintiff suffered a closed head injury, multiple fractured ribs, broken leg, ruptured spleen and other injuries. He was awarded \$75,000.00 in general damages for ongoing residual pain and headaches.

[374] Plaintiff's counsel focused on brain injury cases rather than neck, back and knee soft tissue cases. They ranged from more serious to less serious.

[375] A more serious case was *MacMillan v. Ontario (Minister of Transportation and Communications)* [1998] O.J. No. 2763. The plaintiff Marilyn MacMillan, was 41 at the time of the accident where she suffered a fractured left wrist, laceration of her left knee, laceration of her scalp, a fractured skull, fractured sinuses and a closed head injury. She was in a coma for three or four weeks and in the hospital for a lengthy period of time. She was 51 when the trial took place and "has permanent deficits relating to her attention span, logical sequencing, object recognition, visual memory, verbal memory, social judgment and some motor functions. She will not improve in the future and will require assistance in managing her affairs for the rest of her life".

[376] Mrs. MacMillan was able to do some household tasks including the preparation of simple meals, washing dishes, vacuuming and making beds. She could purchase her own groceries and travel by bus with the assistance of the bus drivers who would give her directions. Her behaviour was often socially unacceptable.

[377] As a result of her head injury she was very aggressive and uninhibited. Her judgment was flawed and her decisions were frequently inappropriate. Her memory was seriously impaired and she occasionally got lost. She had a notable loss of cognitive function, particularly in the areas of concentration, memory and behaviour control. Her peripheral vision to the left was severely reduced and she is physically clumsy.

[378] At paragraph 210 the trial judge assessed general damages and stated "In this case the plaintiff suffered severe injuries which have resulted in serious deficits, which cause her great pain and suffering and loss of enjoyment of life. Her marriage has failed, her ability to function in the community has been diminished and she is no longer employable. I find, however, that the plaintiff's injuries do not fall into the category of the most seriously injured plaintiffs as with the assistance of her family and friends, she can do a great deal to enhance the quality of her life. I therefore assess Marilyn MacMillan's general damages at \$210,000.

[379] Less serious but similar to the present situation was *Fournier v. Stevenson Bros. Warehousing Inc.* [2003] B.C.J. No. 679. The Plaintiff was injured in a single car accident on May 17, 1999. She was a 54-year-old female clerical worker at the time.

[380] Hospital records noted that Ms. Fournier had fractures at the C1, C2 and T4 level. The neurological chart indicated that her Glasgow Coma Scale was normal on May 18. When she arrived on the ward from Emergency, she was described in the nurses' notes

as being drowsy but oriented. She complained of headache and there were indications that she had been nauseous throughout. She also complained of chest pain and having difficulty breathing. She was described as being anxious and it was noted that she continued to be nauseous. She still seemed confused or disoriented on May 20, as she was talking about a dog that died four years before as though it were still alive. She continued to be confused and to speak inappropriately. At times she would become disoriented and complained of continuous headache. She continued to be drowsy but rousable.

[381] The history sheet from the day after admission indicated that Ms. Fournier sustained a mild head injury with subarachnoid hemorrhage along with dental, chest and spinal injuries. Confusion was noted throughout her stay and it was noted that she was still not appropriate on May 20. On May 21, she was noted to be amnesic and to have significant cognitive impairment. It was felt that she would not need formal psychiatric follow-up after discharge, which was on May 28.

[382] Witnesses described the plaintiff as a changed person, lacking in confidence, uncomfortable in any size of crowd, having low self-esteem, and being very forgetful. She had trouble following simple driving directions, making or counting change, had difficulty finding words and would lose her train of thought. She engaged in odd conduct while shopping. For example, on impulse she once bought 70 pounds of blueberries and on another occasion 20 or so sets of towels. Where once she was active, outgoing, ambitious, and decisive, she was now fragile, vulnerable and unable to properly manage day-to-day living on her own.

[383] The expert evidence was set out at paragraphs 45 and 46. In the fall of 2000, “her intellectual abilities were then stated to be low, falling within the borderline to low average range.”

[384] However, in 2002 the assessment revealed intellectual abilities within the average range, which was a substantial improvement from the previous assessment. Neuropsychological testing indicated a moderate level of impairment. This included poor cognitive flexibility, a weak sense of spatial localization and impaired auditory information processing. Motor speed was impaired, possibly due to peripheral factors.

[385] Personality testing revealed very high levels of emotional maladjustment, to the extent that psychological factors probably contributed quite strongly to the overall symptom pattern and, “may also be involved in the neuropsychological profile. She is probably depressed and is highly anxious.”

[386] The trial judge stated “taking into account the nature of Ms. Fournier's injuries and her prognosis, I award the plaintiff the sum of \$185,000 by way of non-pecuniary damages.”

[387] Counsel submitted that this closely resembled Hornick who was once a worker and able to cope with life and who now was fragile, vulnerable and more frail both physically and mentally. He was not the man he once was.

[388] A less serious case was *West v. Cotton*, [1993] B.C.J. No. 394. The 40-year-old plaintiff's vehicle was struck with great force in the rear while parked. He was able to walk away but later collapsed. Reconstruction of the accident established that the roof collapsed and the plaintiff hit his head.

[389] He suffered back, neck and leg injuries. He began to experience psychological symptoms such as memory loss. It was established that he suffered a brain injury as a result of the accident resulting in permanent physical and behavioural dysfunction.

[390] The trial judge concluded that the following conditions resulted from the Plaintiff's head injury, "organic mood syndrome (depressed type), chronic pain syndrome (sharp stabbing pains that arise from a diffuse axonal injury), organic personality syndrome or an organic mood disorder, presenting with irritability and lability of moods, reduction in memory (and in particular an incapacity to lay down new memories), reduction in concentration, reduction in the dexterity and strength of the right hand, reduction in verbal ability and headaches". He awarded the Plaintiff \$150,000.00, which award was upheld on appeal.

[391] *Trayner v. Degroot*, [2002] B.C.J. No. 1179, dealt with a pedestrian who was struck by a motor vehicle. It was a relatively recent trial level decision and was an example of a less severe brain injury case notwithstanding a very serious start point.

[392] A 25-year-old female was struck by a taxi cab. She suffered a very serious brain injury in this case with an original Glasgow Coma Scale reading of 3 at the scene and 11 at the hospital. She recovered from this brain injury very well. After the brain injury she was able to work full time, got married and gave birth to a child. She completed a University degree, went surfing, played soccer, ran and exercised in a gymnasium. She did remarkably well after the very serious brain injury.

[393] The medical opinions and evidence of friends and relatives was that her permanent injury was subtle and was something that would not be noticed by a person who met her casually, yet it restricted her or prevented her functioning at the higher level she was once capable of attaining.

[394] The trial judge stated at paragraph 72 that "in my opinion, a reasonable assessment of Mrs. Charles' claim for general damages for the pain, suffering and loss of amenities of life she has suffered to date, and will suffer in the future is \$120,000.00.

[395] Counsel submitted that Hornick was in a more compromised position than Mrs. Charles.

[396] It is clear that there is a higher range of awards in British Columbia than there is in Ontario. However, I am satisfied on the facts of this case as I heard them and the observations of Hornick that I made during the trial that somewhere between \$75,000.00 and \$150,000.00 would be appropriate. Recent British Columbia cases are at the higher range.

Conclusion:

[397] Having considered all of the evidence in this trial concerning Hornick's ongoing physical, mental and psychological problems, I find he has suffered great pain and suffering and loss of enjoyment of life. His marriage has failed and his ability to function physically and mentally in the community has been diminished. I am satisfied that a fair and reasonable assessment of his general damages for pain, suffering and loss of amenities of life would be \$125,000.00.

What was Whitney Hornick's past-lost income, if any?

[398] Before this question can be answered the following question raised by the defence needs to be answered, "What is the date to which Mr. Hornick's past income loss is to be assessed?"

[399] S. 267.5(1) of the Insurance Act deals with past-lost income. There is a distinction in the measure of loss of income "*before the trial of the action*" and "*after the trial of the action.*" It states:

267.5(1) Despite any other Act and subject to subsection (6), the owner of an automobile, the occupants of an automobile and any person present at the incident are not liable in an action in Ontario for the following damages for income loss and loss of earning capacity from bodily injury or death arising directly or indirectly from the use or operation of the automobile:

1. Damages for income loss suffered in the seven days after the incident.
2. Damages for income loss suffered more than seven days after the incident and **before the trial of the action** in excess of 80 per cent of the net income loss, as determined in accordance with the regulations, suffered during that period.
3. Damages for loss of earning capacity suffered after the incident and **before the trial of the action** in excess of 80 per cent of the net loss of earning capacity, as determined in accordance with

the regulations, suffered during that period. 1996, c. 21, s. 29. [my emphasis added]

[400] “*Before the trial of the action*,” loss of income is determined on the basis of 80% of the net loss calculated after deducting income tax, CPP and UIC. Future loss of earning capacity is assessed on the basis of the gross loss of income without deduction for income tax and other items.

[401] Thus, the court must determine the transition date. This turns on the meaning of the words “*before the trial of the action*” in s. 267.5(1). It was suggested that the words “before the trial of the action” in this section have not been judicially considered.

[402] It was submitted by Mr. Strosberg that the clause “before the trial of the action” should be interpreted as meaning before the last day of the trial when judgment is released and not when counsel give opening statements or when the first piece of evidence is called. He quoted from *Catherwood et al. v. Thompson* [1958] O.R. 326 at p. 331 where Shroeder J.A., speaking for the Court of Appeal, said “*in a general sense, the term ‘trial’ denotes the investigation and determination of a matter in issue between the parties ... advancing through progressive stages from its submission to the court or jury to the pronouncement of judgment.*”

[403] Therefore, the court should calculate past loss of income to the date of judgment and loss of future earning capacity thereafter. As such, the provisions of section 267.5(1) should apply until the date of judgment. If this were not the case, then there would be a hiatus between February 22, 2003, when the hearing commenced, and the date of the pronouncement of judgment which would cause difficulty in calculating collateral benefit credits contemplated by s. 267.8(9). To interpret the word “*trial*” to mean other than the date of judgment would allow Hornick to receive double recovery by not being obliged to account for the monthly Maritime Life benefits received during the period February 22, 2003 to the pronouncement of judgment.

[404] The Rules of Practice state specifically in rule 49.03 that the costs consequences of the rule are triggered by an offer made more than seven days “before the hearing commences.”

[405] Counsel for the Plaintiff obviously took the opposite position and urged me to find that the trial commenced at the start of evidence being introduced and that date established the time frame for calculating the past loss damage quantum.

Discussion:

[406] The commencement of a civil trial seems to depend upon the context for which determination is required. For example, in situations where a juror is disqualified

after having been sworn, the Ontario Court of Appeal has found that the trial started with the opening instructions and addresses.

[407] In the context of Rule 49 and the costs consequences of settlement offers, the majority of Ontario decisions support the view that the trial commences only with the calling of evidence. There has been one decision to the contrary in which the trial judge found the trial commenced when the action was called for trial by a judge sitting in court who was prepared to proceed with the trial.

Court of Appeal:

[408] In *Catherwood v. Thompson*, [1958] O.R. 326 (C.A.) Schroeder J.A. gave one of the first pronouncements on when a civil trial commences. I set out below the complete quote from the court:

In a general sense, the term “trial” denotes the investigation and determination of a matter in issue between the parties before a competent tribunal, advancing through progressive stages from its submission to the court or jury to the pronouncement of judgment. When a trial may be said actually to have commenced is often a difficult question but, generally speaking, this stage is reached when all preliminary questions have been determined and the jury, or a judge in a non-jury trial, **enter upon the hearing and examination of the facts for the purpose of determining the questions in controversy in the litigation.** (p. 331) [my emphasis added]

The complete quote sheds a different light on the meaning of when a civil trial actually commences. This clearly is not supportive of Mr. Strosberg’s position. Further assistance in determining this question can be found from the following decisions.

[409] In *Sauve v. Pokorny*, [1997] O.J. NO. 3332, the Court of Appeal considered when a trial starts in the context of s.108 of the Courts of Justice Act. The jury had been selected and the judge had given his opening instructions, followed by the opening addresses by both counsel. Evidence had not been called. During the lunch break, one of the jurors informed a court official that she had prior knowledge of both parties. The trial judge directed the trial to continue with five jurors. At issue in the appeal was whether the trial judge should have selected another juror.

[410] Goudge J.A. found that the trial judge was correct to continue with only five jurors. The jury had already started to act as a jury by receiving judicial instructions and listening to the opening addresses of counsel.

[411] Morden A.C.J.O. concurred, and found that a trial does not commence with the calling of evidence. The opening instructions and addresses are a part of the trial. He noted:

Although there is no step in a civil jury proceeding which is exactly equivalent to that of putting an accused in charge of the jury in a criminal proceeding, there are stages in a civil proceeding which are parallel to those in a criminal proceeding which are clearly part of a criminal trial, for instance, the opening instructions and addresses. The proceeding is in the hands of the jury during these stages in criminal proceedings.

[412] Weiler J.A. dissented because it was her opinion that for the purposes of s.108, a civil trial begins only when the evidence is heard.

[413] Finally, in *Piller v. Association of Ontario Land Surveyors*, [2002] O.J. No. 2343, Gillese J.A. noted in obiter that “the answer to the question of when a civil trial begins appears to vary depending upon the context in which the question is asked.” No further guidance in the case was provided as to when a civil trial begins.

Trial Level:

[414] In *Kirkpatrick v. Crawford*, [1987] O.J. NO. 2337, Fedak D.C.J. found that in the context of Rule 49, the hearing commenced when evidence was adduced. He concluded at para. 9:

I come to the conclusion that the commencement of hearing as required in R. 49 takes place as soon as evidence is adduced. It is the moment that a jury or a Judge becomes seized with the evidence. Prior to evidence being heard, the jury could be discharged on a settlement without requiring it to agree to a verdict. A jury can be picked before one Judge and a trial take place with another Judge.

[415] Similarly, in *Jonas v. Barma*, [1987] O.J. No. 2344, McDonald D.C.J. held that the hearing or trial does not commence until the first evidence is called. He noted that it is sometimes impractical for the trial to commence immediately after the selection of the jury, and that it would be unjust to prevent a defendant from making an offer to settle during the intervening period.

[416] Matlow J. explicitly disagreed with the *Jonas, supra*, decision. In *Bontje v. Campbell, Roy & Brown Insurance Brokers Inc.*, [1994] O.J. No. 3025. Matlow J. explained that a civil non-jury trial starts before evidence is called:

In my view, for purposes related to the application of Rule 49, a civil non-jury trial commences when the action is called for trial by a judge sitting in court and prepared to proceed with the trial. It is not necessary that any evidence be adduced before the trial, or “hearing”, actually commences.

[417] In *Villeneuve v. Scott*, [1997] O.J. No. 325, MacKinnon J. held that the hearing commenced for the purposes of Rule 49 as soon as evidence is adduced. In this case, the jury was selected on August 26th, 1996, but counsel neither started the opening

address nor called evidence until September 3, 1996. MacKinnon J. found that the settlement offer made August 23rd did comply with the provisions of Rule 49.10(1)(a) and the plaintiff was entitled to solicitor and client costs after the date of the offer.

[418] Finally, in *Capela v. Rush*, [2002] O.J. No. 1527, Sutherland J. canvassed the case law on the issue of when a civil trial commences. He noted that the predominant opinion was that a civil trial begins only when the jury first hears evidence. He observes that although no distinction was made in these cases between jury and non-jury cases, that the language in *Kirkpatrick, supra*, and in *Villeneuve, supra*, was consistent with the view that even in a non-jury case, the hearing does not commence until the court has heard evidence. However, Sutherland J. declined to express an opinion on the commencement of the hearing in non-jury cases. He did find that in jury cases, the trial commences with the calling of evidence.

Conclusion

[419] The commencement of a civil trial depends upon the context of the case. It appears that no matter which case is referred to, once evidence is called the trial has started. In cases involving s.108 of the Courts of Justice Act, the trial starts when the opening instructions and addresses are given.

[420] In cases involving Rule 49 of the Rules of Civil Procedure, the predominant judicial view has been that the trial starts when judge or jury first hears evidence. There has been one case in which an Ontario judge found that the trial started before the calling of evidence. Although this case appears to conflict with the other decisions, it may be reconcilable. Matlow J. did not find that the trial began when the jury was selected. He merely found that the trial commenced when the judge was in court, prepared to proceed with the trial. In most cases, this would be the point at which instructions are given, counsel addresses made or evidence is called.

[421] Here it is clear that the words “before the trial of the action” can only have the common sense interpretation that once the trial starts there is no longer a period of time before the trial of the action as the trial of the action has commenced. I find that for the determination of the past loss income, the critical time point is when counsel address the court in the opening statement and certainly no later than when the first evidence is called and not the date of the judgment.

Past lost income:

[422] Liability for Hornick’s past lost income losses is limited to 80% of net income from 7 days after the accident up the date of the commencement of the trial, February 26, 2003.

[423] Net income includes gross income, less deductions for Employment Insurance, Canada Pension Plan premiums and income tax.

[424] Exhibit 51 was a report from Dilkes, Jeffery and Associates authored by James E. [Jay] Jeffery [Jeffery] and dated February 26, 2003. It was based on a valuation date of February 24, 2003. exhibit 59 was Mr. Wallack's [Wallack] report.

[425] The defence argued that the figures utilized by Jeffery and Wallack were not correct. Jeffery's numbers [\$112,686.00] were different from Mr. Wallack's numbers [\$164,679.00].

[426] After considering their calculations I am satisfied there did not appear to be any acceptable way for either Jeffery or Wallack to fix the income of Hornick as they did in exhibit 51 or 59. The Plaintiff did not lead evidence that established by substantial probabilities that the projected income was as set out by these two men.

[427] A more reasonably probable way to calculate the income would be to take a historical average for the last 4 years that he worked commencing in 1994 up to 1997 when it appears that he would have earned \$31,333.00 according to the submission of the defence. To that figure should be added the appropriate cost of living increase on a yearly basis to get the past-lost income to February 23, 2003. Counsel will work out this amount in accordance with my final directions at the end of this judgment.

What will be Whitney Hornick's lost future income or loss of future earning capacity, if any?

[428] The test concerning how to assess future loss of earning capacity [future income loss] was set out in *Schrump v. Koop* 18 O.R. (2d) 337 The head note summarizes the test easily:

"In the area of the law relating to the assessment of damages for physical injury, though it may be necessary for a plaintiff to prove on the balance of probabilities that the tortious act or omission was the effective cause of the harm suffered, it is not necessary for him to prove that future loss or damage will occur, but only that there is a **reasonable chance of such loss or damage occurring**. Speculative and fanciful possibilities unsupported by expert or other cogent evidence can be removed from the consideration of the trier of fact and should be ignored, whereas **substantial possibilities based on such expert or cogent evidence must be considered**. This principle applies regardless of the percentage of possibility, as long as it is a substantial one, and regardless of whether the possibility is favourable or unfavourable. Thus, future contingencies which are less than probable are regarded as factors to be considered, provided they are shown to be substantial and not speculative: they may tend to increase or reduce the award in a proper case. [my emphasis added]

What age would Hornick work to had he not been injured?

[429] The plaintiff took the position that Hornick was a worker and had worked consistently since leaving high school. He worked in a gas station, in a furnace factory, put in overtime, painted and even drove the company bus. He knew how to work. When he was laid off he did not collect unemployment, he went out and found a tougher job with Miller where he was outside in the cold in the winter and heat in the summer and where he had to deal with big heavy objects and equipment. In between, he had a job at a scrap yard. He was a big strong guy and he wanted to work.

[430] The defence argued that after the crash in December of 1997, Hornick was laid off by Miller Maintenance and it is insufficient for him to say that because he previously worked, he would find a job. The economy changes, he ages and the past does not necessarily portend the future. In addition, Hornick testified that he was not going to look for a job as a truck driver and would not settle for anything less. Further, he did not make any sincere efforts to find employment. After May 29, 1998 Hornick was unemployed and there was no evidence that he would have been able to find a job. Jeffery said that manufacturing jobs were lost in Kent County.

[431] Plaintiff's counsel urged that I look at what a comparable employee at Miller would have made on a yearly basis in 1997 and after. This would be the best comparable to use in making a decision based on the substantial possibilities test applied to the projection of Hornick's future earning capacity. I have already commented on how to compute his income.

[432] I find that notwithstanding Hornick telling Dr. Sweeney he wanted to retire early, the realities concerning a man who has been a labourer [notwithstanding his supervisory position at Duo-Matic] indicate that there was a substantial possibility he would have had to work to age 65.

Contingencies:

[433] The court must take into consideration contingencies which may affect future earning capacity. As Dickson J. said in *Andrews v. Grand & Toy Alberta Ltd.*, [1978] 2 S.C.R. 229 at pp. 253-254:

“(iii) Contingencies: It is a general practice to take account of contingencies which might have affected future earnings, such as unemployment, illness, accidents and business depression. In the *Bisson* case, which also concerned a young quadriplegic, an allowance of 20 per cent was made. There is much support for the view that such a discount for contingencies should be made: see *e.g. Warren v. King; McKay v. Board of Govan School Unit No. 29 of Saskatchewan*. There are, however, a number of qualifications which should be made. First, in many respects, these contingencies implicitly are already contained in an assessment of the projected average level of earnings of the injured person, for one must assume that this figure is a projection with respect to the real world of

work, vicissitudes and all. Second, not all contingencies are adverse, as the above list would appear to indicate. As is said in *Bresatz v. Przibilla*, in the Australian High Court, at p. 544: “Why count the possible buffets and ignore the rewards of fortune?” Finally, in modern society there are many public and private schemes which cushion the individual against adverse contingencies. Clearly, the percentage deduction which is proper will depend on the facts of the individual case, particularly the nature of the plaintiff’s occupation, but generally it will be small: see Stevens, “Actuarial Assessment of Damages: The Thalidomide Case” (1972), 35 M.L.R. 140, at p. 150.

In reducing Andrews’ award by 20 per cent Mr. Justice Kirby gives no reasons. The Appellate Division also applied a 20 per cent reduction. It seems to me that actuarial evidence could be of great help here. Contingencies are susceptible to more exact calculation than is usually apparent in the cases; see Traversy: “Actuaries and the Courts”, 29 Aust. L.J. 557. In my view, some degree of specificity, supported by evidence, ought to be forthcoming at trial.

The figure used to take account of contingencies is obviously an arbitrary one. The figure of 20 per cent which was used in the lower Courts (and in many other cases) although not entirely satisfactory, should, I think, be accepted.”

[434] The negative contingencies that must be considered include Hornick’s degenerative disease at L5-S1, his neck and knee injury, his lack of education, his lack of union protection, his historical periods of unemployment, social assistance and reduced time at Duo-Matic, other illnesses, business depression and his actual termination of employment by Miller Paving on May 29, 1998 and the costs he would have incurred if he were employed.

Conclusion:

[435] I am of the view that the figure of 20% is reasonably fair in all the circumstances of this case.

The calculation:

[436] Exhibit 51 contained the plaintiff’s calculation of future lost earnings. Schedule 1 set out that the age at valuation date was 44.34 as of February 24, 2003. The mortality tables were as set in the Life Tables, Canada, 1995-97. Net discount rates of 2.5 % for the first 15 years and 2.5% thereafter were utilized. Hornick had a life expectancy during the period considered [to age 65] of 19.6 years and the present value of \$1000.00 per year was \$15,470.00 which I accept.

[437] Schedule 2 was an illustration of factors to determine the present value of future losses. The figure for basic earnings needs to be adjusted in accordance with the instructions in the past lost income section and the multiplication extended.

[438] In terms of calculating offsetting annual earnings, I find that the calculation on page 4 of exhibit 51 reflects a substantial possibility of offsetting annual earnings of \$12,467.00 and the lost fringe benefits calculations will be acceptable after the basic earnings are recalculated.

[439] Page 3 set out an estimate of projected basis annual earnings, figures that will have to be recalculated at a later time in order to come to an average actual income in the years 2003 and 2004. Page 4 set out the offsetting [mitigating] annual earnings based on Hornick's ability to work that would be restricted to occupations with minimal physical demands with medically identified limitations that would restrict his vocational choices. I am satisfied that the calculation of \$12,467.00 is appropriate as is the calculation of lost fringe benefits of \$1923.00.

[440] Schedule 2 will have to be recalculated with the appropriate figures at a later date in order to determine total future losses.

Conclusion:

[441] I am aware that there is always a risk of overcompensation and also a risk of under compensation. It is important to note that Hornick only gets one chance to make his case for future lost income. As this aspect is the one that carries the highest monetary award I am satisfied that when the calculation is completed the figure will be fair and reasonable considering the limitations on Hornick referred to on page 4 of exhibit 51. If there are any questions about these calculations I may be spoken to by counsel.

What will be Whitney Hornick's lost handyman capacity to earn income and what would that income be?

[442] This claim was abandoned by the Plaintiff.

What are the credits, if any, to be deducted from Hornick's claims?

[443] Credits are offset before the trial and assignments are assignments of future benefits post judgment. The defence is entitled to credits in accordance with s. 267.8(1) of the Insurance Act. The Plaintiff conceded that the Defendant was entitled to credit for the Statutory Accident Benefits [SABS] as well as Income Replacement Benefits [IRBS], received by Hornick.

[444] In schedule 2 of Wallack's report [exhibit 59] he details the collateral benefits received as follows:

a.	IRBS	\$62,429.00
b.	Disability Benefits to 2,26,03	\$76,357.00
c.	Disability Benefits 2,27,03 – 4,19,04	\$18,532.00
	[during the trial proper]	
	TOTAL	\$157,318.00

[445] Regarding item b. there has an assignment to Maritime Life in the event that Canada Pension Plan benefits are paid the Plaintiff.

[446] An interesting question was raised about what accounting has to be done concerning the money received by the Plaintiff during the trial proper. It was a large amount of money as the trial was delayed several times to accommodate counsel, witnesses and the court.

[447] The question was, should the payment of item c. above be considered a credit. It was not received before the trial as I have found the meaning of those words to be.

[448] The Plaintiff suggested that because he has received the money it should be treated as a credit.

[449] The Defendant argued the section indicated “damages to which a plaintiff is entitled for income loss and loss of future earning capacity shall be reduced by the following amounts” which included the amounts set out in the total of \$157, 318.00, namely SAABS and Long Term Disability [Ltd.] plus sick leave payments of which there were none here. That amount should be set off against any damages awarded for past loss as well as future loss of earning capacity but not against general damages.

[450] The Plaintiff pointed out the decision of *Bannon v McNealey* [1998] O.J. No. 1673 where at page 23, item 2 and 3, the court said;

2. “Where feasible any no fault benefits paid or payable under s. 6 and 7 of the SABs that are deductible from a tort award under 267(1)(a) must be deductible from a head of damage or type of loss **akin** to that for which the no fault benefits were intended to compensate.
3. If the no fault deduction exceeds the amount awarded under the **specific head of damage** that in this circumstance is the income loss, it’s the past

loss to which the no fault benefits can be applied. Then they cannot resort to another portion of the tort award for the balance.” [my emphasis added]

[451] Defence counsel urged that section 2 means it could not be set off against general damages, a specific head of damages. However, past and the future loss of earning capacity are also heads of damage that are akin to each other. Here it is income loss split between past and future situations and each are loss of income in effect. If one suffers an income loss, the loss of income merges in the judgment and there is an offset because at the end of the day there should not be double compensation.

[452] Plaintiff’s counsel disagreed.

Conclusion:

[453] I find that past wage loss and loss of future earning capacity are two different heads of damage. Credits only apply to pre-trial wage loss. Credits are not assignments and the credits in this case are the SABS and the IRBS. They are to be applied to the past wage loss claim and the past wage loss claim only. If the total of the SABS and the IRBS exceeds the past wage loss claim, then pursuant to *Bannon v. McNeeley*, the defence cannot apply those credits towards the future wage loss claim. That is where assignments are taken into account.

[454] The words in item 3, “specific head of damage” do not mean “any head of damage”. The plain meaning of the words in item 3 make set off of overpayment [pre judgment] of any benefits inappropriate if sought to be applied to future loss of earning capacity.

[455] The difficulty in quantifying the past wage loss here is that it cannot be calculated at this time because payments will continue until the date of judgment. The math will have to be done when judgment is rendered.

What happens to Whitney Hornick’s long-term disability benefits?

[456] Plaintiff’s counsel conceded that the defence had the right to request an assignment of future long-term disability payments under s. 267.8(12). It is clear that the section allows conditions to be put in place that are “just”. I am satisfied that because the Defendant truck company is resident in the U.S.A. that the L.T.D. benefits will not be assigned to U.S.A. Truck until the judgment is paid to Hornick. I make no finding as to whether Hornick is totally disabled. Defence counsel had no difficulty with this condition concerning long-term disability benefits.

What happens to Whitney Hornick’s future Statutory Accident Benefits payments?

[457] This matter was not discussed in final argument by counsel and if necessary this will be discussed when we deal with the matters still to be determined below

Matters still to be determined:

Counsel requested that we meet again to discuss the following matters:

1. Pre-judgment interest
2. Costs
3. Disbursements
4. G.S.T.
5. What will happen to future SABS payments.

[458] Counsel may contact the trial coordinator to arrange a date to consider submissions on these matters if necessary.

I am satisfied on the balance of probabilities that the Plaintiffs be successful on their claims as set out above and judgment will issue in accordance with the findings above.

“Original signed by Justice Thomson”
Gordon I. Thomson
Justice

Released: March 29, 2005

COURT FILE NO.: 546/99 (Chatham)

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

WHITNEY HORNICK, PAMELA
HORNICK, DONALD HORNICK by his
litigation guardian WHITNEY
HORNICK, and DYLAN HORNICK by
his litigation guardian WHITNEY
HORNICK

Plaintiffs

- and -

WARREN KOCHINSKY and U.S.A.
TRUCK INC.

Defendants

REASONS FOR JUDGMENT

Thomson J.

Released: March 29, 2005